UNITED STATES DISTRICT COURT

EASTERN DISTRICT OF WASHINGTON

SPOKANE DIVISION

JANE ELIZABETH ROBERTS, *et al,*	*	
	*	
Plaintiffs,	*	
	*	
VERSUS	*	CIVIL ACTION
	*	
SHRINERS HOSPITALS FOR CHILDREN, SHRINERS HOSPITALS FOR CHILDREN – SPOKANE, BEVERLY BOKOVITZ, FRANCES FARLEY, JERRY GANTT, JOHN McCABE, PHILLIP GRADY, PETER BREWER, AND GOV. JAY INSLEE	*	
	*	
Defendants,	*	

* * * * * * * * * * * * *

COMPLAINT
(JURY TRIAL REQUESTED)

NOW INTO COURT, through undersigned counsel, come Plaintiffs, Jane Elizabeth Roberts, *et al*, (hereinafter "Plaintiffs"), who file this Complaint against Defendants, Shriners Hospitals for Children, Inc., Shriners Hospital for Children – Spokane, Beverly Bokovitz, Frances Farley, MD, Jerry Gantt, John McCabe, Phillip Grady, Peter Brewer, and Gov. Jay Inslee (hereinafter "Defendants"), presenting allegations and causes of action as follows:

DESCRIPTION OF CAUSE OF ACTION

This is a §1983 case seeking redress from Defendants for the deprivation of Plaintiffs' Constitutional and federally secured right to refuse an EUA investigational drug without incurring a penalty or loss of benefits to which Plaintiffs were otherwise entitled.

This lawsuit is being brought under 42 U.S.C. §1983 seeking redress for deprivation of rights granted to Plaintiffs by the United States Constitution, 21 U.S.C. §360bbb-3 *et seq* (the EUA

statute), 42 USC §247d-6d *et seq* (the PREP Act), 45 CFR Part 46, 18 U.S.C. §242, ICCPR Treaty, and the common laws of the State of Washington to hold accountable Shriners Hospitals for Children and Shriners Hospitals for Children – Spokane, State Actors at all times pertinent herein, and its PolicyMakers, the Chief Nursing Officer (CNO) Beverly Bokovitz, Chief Medical Officer Frances Farley, MD, Jerry Gantt, John McCabe, Phillip Grady, and Peter Brewer (Shriners and its named executives will be collectively referred to as "Shriners PolicyMakers") and Gov. Jay Inslee, for damages arising out of their unconstitutional, unlawful, malicious, unequal and contractually violative COVID-19 investigational drug mandate. Special laws apply to Governor Inslee and Shriners' vaccine mandates because the FDA defines the drugs at issue as "investigational with no license for any indication." And even though Defendants' mandates were instituted during and in response to a pandemic emergency, as the U.S. Supreme Court noted since the beginning of the pandemic: "**even in a pandemic, the Constitution cannot be put away and forgotten**." *Roman Catholic Diocese of Brooklyn v. Cuomo*, 141 S.Ct. 63, 208 L.Ed.2d 206 (2020).

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I. Introduction

1. In early 2020, the nation and the world faced a novel coronavirus called SARS-CoV-2, which caused the highly contagious disease COVID-19.

2. On January 31, 2020, the Secretary of Health and Human Services (HHS) declared a public health emergency. The President declared a national emergency on March 13, 2020, of which led to the development of investigational new drugs designed to perform as a vaccine from the virus, i.e., cause the body to produce antibodies to the virus so that the person is immune from infection when exposed to the true virus.

3. To implement the nationwide distribution and administration of these investigational new drugs, the U.S. Food and Drug Administration (FDA) issued an Emergency Use Authorization pursuant to 21 U.S.C. 360bbb-3 (Section 564 of the Food, Drug & Cosmetic Act.)

4. The FDA made clear on its website:

> FDA believes that terms and conditions of an EUA issued under Section 564 preempt state or local law, both legislative requirement and common-law duties, that impose different or additional requirements on the medical product for which the EUA was issued in the context of the emergency declared under section 564…In an emergency, it is critical that the conditions that are part of the EUA or an order or waiver issued pursuant to section 564A – those that FDA has determined to be necessary or appropriate to protect public health – be strictly followed, and no additional conditions be imposed.

5. In August 2020, the Centers for Disease Control (CDC) published the transcript of a meeting of the Advisory Committee on Immunizations and Respiratory Diseases, at which Dr. Amanda Cohn stated (@1:14:40):

> I just wanted to add that, just wanted to remind everybody, that **under an Emergency Use Authorization, an EUA, vaccines are not allowed to be mandatory**. So, early in the vaccination phase, individuals will have to be **consented** and **they won't be able to be mandated.** (Emphasis added)

6. In August 2021, Gov. Inslee usurped the authority of the United States Congress by issuing an official proclamation in defiance of federal law when he mandated the use of investigational new drugs by healthcare workers. Additionally, Gov. Inslee engaged in outrageous tyrannical conduct by mandating that healthcare facilities deny employment to healthcare workers who exercised their federally secured right to refuse investigational drugs.

7. Shriners PolicyMakers decided that the suffering of the few was justified by the windfall such suffering had on Shriners' financial bottom line. Thus, Shriners PolicyMakers prescribed its own "required conditions" in defiance of Congress and the rights of individuals under Defendants' authority as secured by the Constitution.

8. In September of 2021, Shriners PolicyMakers issued a despicable illegal mandate that shocked the conscience. During the height of the pandemic, when hospitalization rates soared, and SARS-CoV-2 variants abounded, and in direct contravention to federal law governing investigational drugs, Shriners PolicyMakers subjected a workforce of more than 16,000 people to investigational drug use under threat of penalty and outside of their free will and voluntary consent. Should those individuals not comply with Shriners PolicyMakers' fraudulent usurpation of authority, they would be segregated, penalized, humiliated, terminated, and denied unemployment benefits, thus depriving Plaintiffs of their Constitutional and federally secured right to refuse an investigational drug without penalty.

9. Gov. Inslee used his office as official cover in hopes of obtaining for himself immunity from liability in future legal actions. Similarly, attempting to hide behind the PREP Act as a liability cover, Shriners PolicyMakers willfully chose to engage in violations of federal law.

II. Jurisdiction and Venue

10. This Court has original jurisdiction under 28 U.S.C. §§ 1331 and 1343.

11. The civil rights portions of this action raise federal questions under the Spending Clause and the Fourteenth Amendment to the U.S. Constitution.

12. This Court has original jurisdiction under 42 U.S.C. §§ 1983 and 1988.

13. This Court has the authority to award costs and reasonable attorney's fees under 42 U.S.C. § 1988.

14. This court has supplemental jurisdiction over Plaintiffs' state law claims.

15. This Court has personal jurisdiction over Defendants as they are domiciled within this Court's jurisdictional boundaries.

16. This Court has subject matter jurisdiction over the parties because all acts complained of herein were committed by Defendants in the State of Washington and caused damage and/or deprivation to the Plaintiffs listed herein.

17. Venue is proper in this court because all events underlying the claims in this Complaint occurred in the State of Washington, which is situated within this Court's jurisdiction, and all Defendants reside in the State of Washington.

III. Plaintiffs

18. The following individuals are plaintiffs herein:

18.1. Plaintiff, Jane Elizabeth Roberts, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.2 Plaintiff, Jon Alleman, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.3. Plaintiff, Michelle Andrews, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.4. Plaintiff, Ginger Bennett, is an adult individual who all times pertinent resided in

the State of Washington, and was previously an employee of Shriners in Washington.

18.5. Plaintiff, Lani Laganowski, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.6. Plaintiff, Inga Miller, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.7 Plaintiff, Francisco Oquendo, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.8. Plaintiff, Erin Palmer, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.9. Plaintiff, Michelle Richardson, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.10. Plaintiff, Peter Springs, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.11. Plaintiff, Kathy Wold, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

18.12. Plaintiff, Julia Zelepukhin, is an adult individual who all times pertinent resided in the State of Washington, and was previously an employee of Shriners in Washington.

IV. Defendants

19. The following are named as defendants herein:

19.1. Defendant, Shriners Hospitals for Children, Inc., is a charitable 501C(3) non-profit corporation incorporated in the State of Colorado and headquartered in Tampa, Florida.

19.2. Defendant, Shriners Hospital for Children – Spokane, at all times pertinent, was a subsidiary of Shriners Hospitals for Children, was the employer of Plaintiffs, and was

authorized to do and doing business in Spokane, WA.

19.3. Defendant, Beverly Bokovitz, was at all times pertinent, the Chief Nursing Officer and PolicyMaker of Shriners and was aware and responsible for duties owed to Plaintiffs under the organization's FWA, IRB, CDC COVID-19 Vaccination Program Provider Agreement on behalf of Shriners Hospitals for Children. Ms. Bokovitz is named as a defendant in her official and individual capacities.

19.4. Defendant, Frances Farley, MD, was at all times pertinent, the Chief Medical Officer and PolicyMaker of Shriners and was aware and responsible for duties owed to Plaintiffs under the organization's FWA, IRB, CDC COVID-19 Vaccination Program Provider Agreement on behalf of Shriners Hospitals for Children. Dr. Farley is named as a defendant in her official and individual capacities.

19.5 Defendant, Jerry Gantt, was at all times pertinent, the Chairman of the Board of Trustees and PolicyMaker of Shriners Hospitals for Children, is an individual of the full age of majority and a resident of Houston, Texas, and was a signatory to Shriners Hospitals for Children's COVID-19 policy. Mr. Gantt is named as a defendant in his official and individual capacities.

19.6 Defendant, John P. McCabe, was at all times pertinent, the Executive Vice President and Chief Operating Officer and PolicyMaker of Shriners Hospitals for Children, is an individual of the full age of majority and a resident of Florida, and was a signatory to Shriners Hospitals for Children's COVID-19 policy. Mr. McCabe is named as a defendant in his official and individual capacities.

19.7 Defendant, Phillip Grady, was at all times pertinent, the Vice President of Hospital Operations and PolicyMaker of Shriners Hospitals for Children, is an individual of the full age of

majority and a resident of Florida, and was a signatory to Shriners Hospitals for Children's COVID-19 policy. Mr. Grady is named as a defendant in his official and individual capacities.

19.8 Defendant, Peter Brewer, was at all times pertinent, the Hospital Administrator and PolicyMaker of Shriners Hospitals for Children – Spokane, is an individual of the full age of majority and a resident of Washington and executed and enforced Shriners Hospitals for Children's COVID-19 policy. Mr. Brewer is named as a defendant in his official and individual capacities.

19.9. Defendant Jay Inslee is the Governor of the State of Washington. Mr. Inslee is named as a defendant in his official and individual capacities.

V. History and Facts

20. Plaintiffs make no assertions regarding whether it is lawful for a public or private entity to mandate a ***licensed*** vaccine. Plaintiffs' allegations herein only relate to Defendants' mandating the use of drugs, biologics, and devices under 21 U.S.C. §360bbb-3 (the EUA statute) and the PREP Act.

21. Plaintiffs adamantly assert that an individual has the absolute Constitutional and federal statutory right to refuse the administration of an Emergency Use Authorization (EUA) drug (e.g., Pfizer BioNTech COVID-19 Vaccine), biologic, or device (e.g., EUA testing articles and masks) or a "covered countermeasure" under PREP Act immunity without incurring a penalty or losing a benefit to which they are otherwise entitled and that such a right is not dependent upon a person seeking a religious or medical exemption.

22. Because the EUA statute was created to allow the Secretary of HHS to authorize the use of a product for a purpose for which it is not already licensed, 21 U.S.C. §360bbb-3 products fall under the investigational or experimental classification by statute.[1]

[1] 21 U.S.C. §360bbb-3(a)(2)(A) and (B)

23. This classification was illustrated in the May 10, 2021, Scope of Authorization letter issued to Pfizer, Inc. wherein the FDA advised Pfizer that its product is "an investigational vaccine not licensed for any indication." The same is true for the Moderna and Jannsen injections.

24. Because EUA products are, by definition, used only during times of emergency, the laws regulating these products are not litigated as much as more commonly used statutes, so a brief recitation of the origin and history of these laws is in order.

25. The laws regulating the investigational new drug (IND) industry were largely created after Senator Edward Kennedy held live hearings in 1973 detailing the industry's abuses against the American people. In 1974, Congress enacted the National Research Act[2] in response to those hearings, establishing laws, regulations, and mandatory guidelines to protect Americans from future abuses. However, the industry has been internally regulated and enforced via various federal and state agencies since 1974. This internal enforcement has denied the judiciary from acquiring knowledge of the laws discussed herein. To ensure Plaintiffs secure equitable justice, the court is provided with an understanding of those laws that have been so well hidden from so many people during the COVID-19 pandemic.

26. The 1974 National Research Act established the National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research[3] (hereinafter referred to as the "Commission").

27. Congress required the Commission to:

A. "conduct a comprehensive investigation and study to identify the basic ethical principles which should underlie the conduct of biomedical and behavioral research involving human subjects,"

[2] Public Law 93-348-July 12, 1974 National Research Act

[3] Title II of the National Research Act - https://www.govinfo.gov/content/pkg/STATUTE-88/pdf/STATUTE-88-Pg342.pdf

B. "develop guidelines which should be followed in such research to assure that it is conducted in accordance with such principles," and

C. "make recommendations to the [HHS] Secretary" for "such administrative action as may be appropriate to apply such guidelines to biomedical and behavioral research conducted or supported under programs administered by the Secretary."

28. Congress further required the Commission to consider "the nature and definition of informed consent in various research settings."[4]

29. On April 18, 1979, the Commission published its findings in the Federal Register in a report titled, "The Belmont Report."[5]

A. The Belmont Report

30. The Belmont Report outlined what the Commission considered "the nature and definition of informed consent" as follows:

A. "An autonomous person is an individual capable of deliberation about personal goals and acting under the direction of such deliberation. To respect autonomy is to give weight to autonomous persons 'considered opinions and choices while refraining from obstructing their actions…" (Emphasis added);

B. "To show lack of Respect for an autonomous agent is to repudiate that person's considered judgments, to deny an individual the freedom to act on those considered judgments…"(Emphasis added);

C. "Respect for persons requires subjects, to the degree that they are capable, be given the opportunity to choose what shall or shall not happen to them. This opportunity is provided when adequate standards for informed consent are satisfied" (Emphasis added).

[4] National Research Act Title II - PROTECTION OF HUMAN SUBJECTS OF BIOMEDICAL AND BEHAVORIAL RESEARCH Part A Section 202. (a)(1)(B)(iv)

[5] The National Commission for the Protection of Human Subjects of Biomedical and Behavioral Research. - Belmont Report. Colorado, DC: U.S. Department of Health and Human Services,1979

31. The Belmont Report defined those adequate standards of informed consent as follows:

> A. An agreement to participate in research constitutes valid consent only if voluntarily given. This element of informed consent requires conditions free of coercion and undue influence; (Emphasis added)
>
> B. Coercion occurs when an overt threat of harm is intentionally presented by one person to another in order to obtain compliance;
>
> C. Undue influence, by contrast, occurs through an offer of an excessive, unwarranted, inappropriate, or improper reward or other overture in order to obtain compliance. Also, inducements that would ordinarily be acceptable may become undue influences if the subject is especially vulnerable;
>
> D. Unjustifiable pressures usually occur when persons in positions of authority or commanding influence -- especially where possible sanctions are involved -- urge a course of action for a subject," (emphasis added), and;
>
> E. …undue influence would include actions such as manipulating a person's choice through the controlling influence of a close relative and threatening to withdraw health services to which an individual would otherwise be entitled.

32. The Commission determined that if an individual is under outside pressure to participate in an investigational medical activity, then obtaining that individual's informed consent was legally impossible.

33. Congress mandated in the National Research Act that "[i]f the Secretary determines that administrative action recommended by the Commission should be undertaken by him, he shall undertake such action as expeditiously as is feasible."

34. Congress required the HHS Secretary to act upon the Commission's recommendations as outlined in the Belmont Report by establishing regulations to protect humans

involved in biomedical research activities. **Therefore, given the complexity, the intent of Congress was not to draft those laws but to allow the HHS Secretary to promulgate regulations on its behalf** to protect humans involved with investigational drugs. Therefore, these regulations are unique in that they were expressly requested by Congress to fulfill the intent of Congress via the National Research Act.

35. In the early 1980s, HHS acted upon the Commission's recommendations, stating, "Based on the Belmont Report and other work of the National Commission, HHS revised and expanded its regulations for protecting human subjects…The HHS regulations are codified at 45 Code of Federal Regulations (CFR) 46, subparts A through D."[6]

B. 45 CFR Part 46

36. 45 CFR Part 46 is entitled, "Protection of Human Subjects." Subpart A is entitled, "Basic HHS Policy for Protection of Human Research Subjects" and establishes that (a) the policy (for protection of human research subjects) "applies to **all research**[7] involving human subjects conducted, supported, or otherwise subject to regulation by any Federal department or agency..." (Emphasis added).[8]

37. HHS designed a very broad definition of research when, at 45 CFR § 46.102 (Definitions): "Research means a systematic investigation, including research development, testing, and evaluation, designed to develop or contribute to generalizable knowledge. Activities that meet this definition constitute research for purposes of this policy, whether or not they are

[6] 45 CFR 46 FAQs. HHS.gov. Published 2018. Accessed May 18, 2023. https://www.hhs.gov/ohrp/regulations-and-policy/guidance/faq/45-cfr-46/index.html

[7] Research under 45 CFR Part 46 includes clinical trials but is not limited in scope to only clinical trials. College students studying medical charts of patients constitutes "research" requiring 45 CFR Part 46 adherence.

[8] 45 CFR 46.101(a)

conducted or supported under a program that is considered research for other purposes"[9] (emphasis added). Research under this policy includes medical chart reviews by students or periodic studies of medical products under 21 U.S.C. §360bb-3 authorization.[10]

38. A "human subject" is broadly defined as (1) a living individual, (2) from whom data is obtained and used,[11] and (3) from whom identifiable private information is known.[12]

39. HHS regulations define[13] the term "human subject" at 45 CFR 46.102(e) as follows:

> (1) ***Human subject*** means a living individual about whom an investigator (whether professional or student) conducting research:
>
> > (i) Obtains information or biospecimens through intervention or interaction with the individual, and, uses, studies, or analyzes the information or biospecimens; or
> >
> > (ii) Obtains, uses, studies, analyzes, or generates identifiable private information or identifiable biospecimens.
>
> (2) ***Intervention*** includes physical procedures by which information or biospecimens are gathered (*e.g.,* venipuncture) and manipulations of the subject or the subject's environment that are performed for research purposes.
>
> (3) ***Interaction*** includes communication or interpersonal contact between investigator and subject.
>
> (4) ***Private information*** includes information about behavior that occurs in a context in which an individual can reasonably expect that no observation or recording is taking place, and information that has been provided for specific purposes by an individual and that the individual can reasonably expect will not be made public (e.g., a medical record).

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[9] 45 CFR 46.102(l)

[10] https://www.hhs.gov/ohrp/sites/default/files/human-subject-regulations-decision-charts-2018-requirements.pdf

[11] 45 CFR 46.102(e)(1)(i)

[12] 45 CFR 46.102(e)(1)(ii)

[13]" Coded Private Information or Biospecimens Used in Research (2018)." HHS.gov. Published January 19, 2018. https://www.hhs.gov/ohrp/coded-private-information-or-biospecimens-used-research.html#:~:text=Identifiable%20private%20information%20is%20private,is%20associated%20with%20the%20information (Last accessed June 5, 2023)

> (5) ***Identifiable private information*** is private information for which the identity of the subject is or may readily be ascertained by the investigator or is associated with the information.
>
> (6) ***An identifiable biospecimen*** is a biospecimen for which the identity of the subject is or may readily be ascertained by the investigator or is associated with the biospecimen. (Emphasis in original.)

40. Congress drafted broad definitions for "research" and "subjects" to comply with the recommendations of the Belmont Report, which declared that "the general rule is that if there is any element of research in an activity, that activity should undergo review (third-party review to ensure the health and rights of involved individuals are protected) for the protection of human subjects"[14] (emphasis added).

41. Therefore, if individuals are administered an investigational medical product and their private identifiable information is collected along with the details about their interaction with the product, and that information is monitored, studied, or analyzed for purposes of adding to the generalizable knowledge of the product, then the activity meets the definition of "research," requiring 45 CFR Part 46 compliance when the federal government is involved.

42. HHS ensured that all research activities involving the federal government must comply with Belmont Report's ethical requirements: (1) "Department or agency heads retain final judgment as to whether a particular activity is covered by this policy, and this judgment shall be exercised consistent with the ethical principles of the Belmont Report"[15] (emphasis added), (2) if

[14] The Belmont Report Part A: Boundaries Between Practice & Research. "Research and practice may be carried on together when research is designed to evaluate the safety and efficacy of a therapy. This need not cause any confusion regarding whether or not the activity requires review; the general rule is that if there is any element of research in an activity, that activity should undergo review for the protection of human subjects."

[15] 45 CFR § 46.101(c)

the activity is considered exempt from the policy, then "the alternative procedures to be followed are consistent with the principles of the Belmont Report."[16]

43. Congress expressly prohibits the federal government from administering an investigational product to an individual without complying with the Belmont Report's ethical principles and 45 CFR § 46.101, et *seq*.

44. Placing an individual under a "sanction" for refusing an EUA drug, biologic, or device patently violates the ethical principles of the Belmont Report.

45. The intent of Congress was to give the Belmont Report the force of law through 45 CFR § 46.101, et *seq*. and the Federal Wide Assurance agreement (see discussion, *infra*) for the explicit purpose of protecting humans when they are offered a federally funded EUA investigational product.

46. To further protect Americans from medical research abuses in the future, Congress declared that, "Federal funds administered by a Federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied."[17] (45 CFR § 46.122)

47. Moreover, Congress also prohibited the United States Military from abusing individuals again by enacting 10 U.S.C. § 980(a), which provides in pertinent part, "Funds appropriated to the Department of Defense may not be used for research involving a human being as an experimental subject unless — (1) the informed consent of the subject is obtained in advance."

[16] 45 CFR § 46.101(i)

[17] All COVID-19 EUA drugs and their administration have been fully funded by the federal government, requiring 45 CFR Part 46 adherence.

48. Therefore, pursuant to 45 CFR § 46.101, et *seq*., "research" occurs when an individual is administered an investigational drug, the individual's private identifiable information is known, and data collected regarding their interaction with the drug is added to the generalizable knowledge about the drug.

49. The COVID-19 CDC Vaccination Program is a research activity requiring 45 CFR § 46.101, et *seq*. compliance as well as each COVID-19 EUA's Scope of Authorization. (See *infra*)

50. At no time may the federal government offer or administer an investigational medical product to an individual if their "legally effective informed consent" is not obtained in advance.

C. Legally Effective Informed Consent

51. 45 CFR § 46.116 sets forth the Belmont Report's "adequate standards" of informed consent[18], and they include, but are not limited to:

> (a)(1) Before involving a human subject in research covered by this policy, an investigator shall obtain the legally effective informed consent of the subject or the subject's legally authorized representative; (Emphasis added)
>
> (a)(2) An investigator shall seek informed consent only under circumstances that provide the prospective subject or the legally authorized representative sufficient opportunity to discuss and consider whether or not to participate and that minimize the possibility of coercion or undue influence; (Emphasis added)
>
> (a)(3) The information that is given to the subject or the legally authorized representative shall be in language understandable to the subject;
>
> (a)(4) The prospective subject or the legally authorized representative must be provided with the information that a reasonable person would want to have in order to make an informed decision about

[18] The Belmont Report and 45 CFR §46.116 contain the only definition for what Congress deems legally effective informed consent. Therefore, when statutes explicitly or implicitly mandate a person to give their legally effective informed consent, these definitions must be understood as the intent of Congress for compliance purposes.

whether to participate, and an opportunity to discuss that information;

(a)(5) Informed consent must begin with a concise and focused presentation of the key information that is most likely to assist a prospective subject or legally authorized representative in understanding the reasons why one might or might not want to participate in the research;

(a)(6) No informed consent may include any exculpatory language through which the subject or the legally authorized representative is made to waive or appear to waive any of the subject's legal rights;

(a)(7) For research involving more than minimal risk, an explanation as to whether any compensation and an explanation as to whether any medical treatments are available if injury occurs…;"

(a)(8) A statement that participation is voluntary, refusal to participate will involve no penalty or loss of benefits to which the subject is otherwise entitled, and the subject may discontinue participation at any time without penalty or loss of benefits to which the subject is otherwise entitled" (Emphasis added).

52. Legally Effective Informed Consent, according to the Belmont Report, can be broken down into its basic formula as:

A. the individual must not be under outside pressure to participate,

B. the only reason an individual participates is that he or she believes the product may benefit their personal health goals, and

C. the conditions of 1 and 2 are established before the individual participates in the investigational product.

53. Only when authorities comply with 45 CFR § 46.101, et *seq*. and the ethical principles of the Belmont Report can an opportunity exist for an individual to give their legally effective informed consent according to 45 CFR § 46.116(a)(1).

54. Informed Consent must be legally effective and prospective, according to HHS.

55. 45 CFR Part 46 applies to all federal agencies, departments, and the military (45 CFR § 46.101(a)). Additionally, twenty federal agencies incorporated 45 CFR Part 46 specifically into their regulatory framework.[19]

56. Through the Federal Wide Assurance (FWA) agreement (see *infra*), all U.S. States and Territories (i.e., state health agencies have FWA agreements) have agreed to obtain the legally effective informed consent of individuals when involving them in investigational medical products.

57. Consensual medical experimentation involving investigational medical products can only exist under conditions that ensure individuals are free from outside pressures to participate.

58. Therefore, individuals have the explicit right to refuse an investigational drug, biologic, or device without incurring a penalty or loss of benefits to which they are otherwise entitled.

59. When Defendants penalized Plaintiffs for refusing to inject a 21 U.S.C. §360bbb-3 investigational drug into their bodies, Defendants breached their fiduciary and statutory duties to obtain Plaintiffs 'legally effective informed consent. (See, *infra*)

D. COVID-19 Research Activities

60. The State of Washington and Shriners PolicyMakers are in a symbiotic relationship to conduct 45 CFR § 46.101, et *seq*. research activities pertaining to COVID-19 EUA drugs, biologics, and devices on behalf of the federal government. Moreover, they are in a symbiotic

[19] https://www.hhs.gov/ohrp/regulations-and-policy/regulations/common-rule/index.html

relationship to obtain legally effective informed consent from individuals offered participation in those experimental medical products.

61. The federal government's Executive Branch purchased all COVID-19 EUA drugs (see, *infra*) and, in conjunction with HHS[20] and the CDC, developed research activities that States and CDC Vaccination Program Providers must conduct on its behalf.

62. Drugs, biologics, and devices authorized under 21 U.S.C. §360bbb-3 (see discussion, *infra*) are classified by the FDA as investigational (experimental)[21],[22] according to their labeling. They have no legal indication to treat, cure, or prevent any disease according to their labeling.

63. Moreover, if a product is already licensed by the FDA for the intended use under the declared emergency, the FDA is prohibited from issuing an EUA. (21 U.S.C. §360bbb-3(c)(3))

64. The only COVID-19 drugs made available to Plaintiffs are classified by the FDA as investigational new drugs. No FDA-licensed COVID-19 vaccines have been introduced into commerce for general commercial marketing since the declaration of the pandemic in March 2020, through the filing of this Complaint.

65. A "marketed drug" is not the same as an "investigational drug."

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[20] The EUA Scope of Authorization assigns research activities to the person acting on behalf of the manufacturer of the drug (the federal government who purchased all of the inventory), and to "emergency stakeholders," and "health care providers."

[21] Investigational new drug means, "A substance that has been tested in the laboratory and has been approved by the U.S. Food and Drug Administration (FDA) for testing in people…Also called experimental drug, IND, investigational agent, and investigational drug." NCI Dictionary of Cancer Terms. National Cancer Institute. Published 2023. Accessed June 25, 2023. https://www.cancer.gov/publications/dictionaries/cancer-terms/def/investigational-new-drug

[22] 21 CFR 312.3 21 CFR 312.3 (Definitions and Interpretations): See "Investigational new drug" and "Clinical investigation" Note that "clinical investigation" is distinct from "clinical trial." While all clinical trials are clinical investigations, not all clinical investigations are clinical trials.

66. A "marketed drug" is one that is licensed by the FDA for general commercial marketing and approved with an indication and usage for the treatment of a particular disease, which, via federal statute, EUA medical countermeasure products must not be. (See 21 USC 355a, *et seq*, 21 USC 360bbb-3(a)(2)(a,b))

67. Investigational new drugs are legally regulated entirely differently than licensed drugs. The FDA declared in its August 23, 2021 EUA to Pfizer that "Pfizer-BioNTech COVID-19 Vaccine" drug is legally distinct from its licensed "COMIRNATY" drug[23].

68. The distinction lies within the drug's classifications as assigned to them by the FDA. Those distinctions have significant legal consequences for the end user. (See discussion, *infra*)

69. EUA drugs, by their statutory definitions, are not licensed by the FDA for general commercial marketing and have no legal indication to treat, cure, or prevent any known disease.

70. Investigational drug "means a new drug or biological drug that is used in a clinical investigation." (21 CFR 312.3 "Investigational new drug")

71. Clinical investigation "means any experiment in which a drug is administered or dispensed to, or used involving, one or more human subjects. For the purposes of this part, an experiment is any use of a drug except for the use of a marketed drug in the course of medical practice." (21 CFR 312.3 "Clinical investigation") (Emphasis added).

[23] See Exhibit B, FDA EUA Letter to Pfizer, August 23, 2021

72. Only the FDA is authorized by Congress to assign a drug, biologic, or device its classification for purposes of regulation.

73. Drugs are governed by their classification according to their labeling and not by their formulation.

74. Congress explicitly enacted laws governing investigational new drugs to prevent the executive branch from continuing its history of abusing the rights of individuals who participate in investigational medical products.

75. On December 11, 2020, the FDA issued to Pfizer-BioNTech the first COVID-19 EUA for its investigational drug (officially named Pfizer-BioNTech COVID-19 Vaccine[24]), and the FDA confirmed that Pfizer's product "is an investigational vaccine not licensed for any indication."[25]

76. On December 18, 2020, the FDA issued to ModernaTX, Inc., an EUA for its investigational drug (officially named Moderna COVID-19 Vaccine), and the FDA confirmed that Moderna's product "is an investigational vaccine not licensed for any indication."[26]

[24] *Id*. The FDA improperly allowed Pfizer to add the word "Vaccine" to its investigational name. The court should not confuse this name to mean the drug's legal indication. Pfizer-BioNTech COVID-19 Vaccine is an investigational drug having no legal indication to treat, cure, or prevent any known disease. The FDA classified the drug as an "investigational new drug."

[25] 86 Fed.Reg. 5200, Jan. 19, 2021

[26] 86 Fed.Reg. 5211, Jan. 19, 2021

77. On February 27, 2021, the FDA issued to Janssen Biotech, Inc., an EUA for its investigational drug (officially named Janssen COVID-19 Vaccine), and the FDA confirmed that Janssen's product "is an investigational vaccine not licensed for any indication."[27]

78. 21 U.S.C. §360bbb-3 requires the Secretary of HHS to "[a]ppropriate conditions for the monitoring and reporting of adverse events associated with the emergency use of the product (research activity)."

79. The Secretary establishes the conditions under which the research activities will occur in each EUA letter, known as the Scope of Authorization.

80. As an example, on January 19, 2021[28] the Secretary established mandatory conditions that Pfizer and emergency stakeholders (distributors, manufacturers, etc.) must follow, which involve 45 CFR 46 research activities.

81. Under the EUA's "Conditions of Authorization," the Secretary mandates in part:

* * *

F. Pfizer Inc. will report to Vaccine Adverse Event Reporting System (VAERS):

- Serious adverse events
- Cases of Multisystem Inflammatory Syndrome in children and adults
- Cases of COVID-19 that result in hospitalization or death, that are reported to Pfizer, Inc.

[27] 86 Fed.Reg. 28608, May 27, 2021

[28] Authorizations of Emergency Use of Two Biological Products During the COVID-19 Pandemic; Availability. Federal Register. Published January 19, 2021. Accessed June 7, 2023. https://www.federalregister.gov/documents/2021/01/19/2021-01022/authorizations-of-emergency-use-of-two-biological-products-during-the-covid-19-pandemic-availability

G. Pfizer Inc. must submit to Investigational New Drug application (IND) number 19736 periodic safety reports at monthly intervals, within 15 days after the last day of a month…Each periodic safety report is required to contain descriptive information which includes:

- A narrative summary and analysis of adverse events submitted during the reporting interval, including interval and cumulative counts by age groups, special populations (e.g., pregnant women), and adverse events of special interest.
- Newly identified safety concerns in the interval.

* * *

N. Pfizer Inc. will conduct post-authorization observational study(ies) to evaluate the association between Pfizer-BioNTech COVID-19 Vaccine and a pre-specified list of adverse events of special interest, along with deaths and hospitalizations, and severe COVID-19. The study population should include individuals administered the authorized Pfizer-BioNTech COVID-19 Vaccine under this EUA in the general U.S. population (16 years of age and older), populations of interest such as healthcare workers, pregnant women, immunocompromised individuals, subpopulations with specific comorbidities.

* * *

T. Vaccination providers administering Pfizer-BioNTech COVID-19 Vaccine must report the following information…to VAERS…:

- Serious adverse events
- Cases of Multisystem Inflammatory Syndrome in children and adults
- Cases of COVID-19 that result in hospitalization or death

82. VAERS reported 1,562,008 entries from December 2020 through May 26, 2023, including 35,272 deaths (1.6 per hour) and 263,462 (12.11 per hour) serious injuries. These numbers demonstrate historical entries for a drug and the vast involvement of the medical community to add to the "generalizable knowledge" of the product.

83. Healthcare providers and Pfizer, Moderna, and Janssen must identify the person receiving the product, monitor their involvement with the product, and report whether or not they had an adverse reaction to the product for the express purpose of adding to the generalizable knowledge of the product.

84. COVID-19 drug manufacturers and government agencies use collected data to add to the generalizable knowledge about the product. These conditions meet 45 CFR 46, FWA, and the Belmont Report definitions of research activities.

85. The CDC Provider Agreement (see discussion, *infra*), EUA authorizations, and CDC's Advisory Committee on Immunization Practices (ACIP) recommendations demonstrate how the nationwide COVID-19 vaccination program is to be systematically investigated.

86. The federal government purchased all COVID-19 drugs and created the CDC COVID-19 Vaccination Provider Agreement for the administration of its property to individuals desiring to participate in the product. The Provider Agreement establishes additional research activities that Defendants must conduct on the government's behalf and "must administer COVID-19 Vaccine in accordance with all requirements and recommendations of CDC and CDC's Advisory Committee on Immunization Practices (ACIP)."

87. ACIP's Morbidity and Mortality Weekly Report from September 2021 confirms that in addition to "initial clinical trial data, ACIP…considered…real-world vaccine effectiveness studies, and post-authorization vaccine safety monitoring," information came from entities that executed the CDC Vaccine Provider Agreement and submitted the below-described information because the ONLY way to have authority to administer the COVID-19 Vaccines is by executing

the CDC Vaccine Provider Agreement.[29] The use of this information by ACIP demonstrates how the data collected "contributes to generalizable knowledge."

88. The ACIP recommendations[30] referenced in Footnote 1 of the CDC Provider Agreement[31] instruct Defendants to:

A. Provide an EUA Fact Sheet to potential recipients before being administered the drug.

B. Counsel potential vaccine recipients about expected systemic and local reactogenicity.

C. Follow additional clinical considerations, including details of administration and use in special populations (e.g., persons who are pregnant or immunocompromised or who have severe allergies) based on advice from the CDC (https://www.cdc.gov/vaccines/covid-19/info-by-manufacturer/ pfizer/clinical-considerations.html)

D. Adverse events that occur in a recipient after receipt of COVID-19 vaccine should be reported to the Vaccine Adverse Events Reporting System (VAERS).

E. Report vaccination administration errors, serious adverse events, cases of multisystem inflammatory syndrome, and cases of COVID-19 that result in hospitalization or death after administration of COVID-19 vaccine under EUA.

F. Report any clinically significant adverse event, whether or not it is clear that a vaccine caused the adverse event.

[29] ACIP, Morbidity and Mortality Weekly Report, "Use of Pfizer-BioNTech COVID-19 Vaccine in Persons Aged ≥ 16 Years: Recommendations of the Advisory Committee on Immunization Practices – United States, September 2021", Vol.70, No.38, p. 1344.

[30] *Id*., at 1347.

[31] The CDC Provider Agreement, at p.2, makes the ACIP Recommendations mandatory by the following language: "This agreement expressly incorporates all recommendations, requirements, and other guidance that this agreement specifically identifies through footnoted weblinks. Organization must monitor such identified guidance for updates. Organization must comply with such updates." See Exhibit A, CDC COVID-19 Vaccination Program Provider Agreement (hereinafter "Provider Agreement").

G. Inform vaccine recipients about V-Safe, the CDC's vaccine safety monitoring system that the CDC says "helps us monitor the safety of COVID-19 vaccines for everyone."[32]

89. The CDC Provider Agreement further instructs Defendants:

A. Within 24 hours of administering a dose of COVID-19 Vaccine, record in the vaccine recipient's record and report required information to the relevant state, local or territorial public health authority.

B. Submit Vaccine-Administration Data through either (1) the immunization information system (IIS) of the state or local territorial jurisdiction or (2) another system designated by CDC according to CDC documentation and data requirements.

C. Organization must preserve the record for at least 3 years following vaccination, or longer if required by state, local, or territorial law. Such records must be available to any federal, state, local, or territorial public health department to the extent authorized by law.

D. Report the number of doses of COVID-19 Vaccine that were unused, spoiled, expired, or wasted as required by the relevant jurisdiction.

E. Provide a completed COVID-19 vaccination record card to every COVID-19 vaccine recipient.

90. Based on the detailed, organized, and methodical way HHS and the CDC structured the nationwide COVID-19 Vaccination Program, it meets the criteria for "a systematic investigation…designed to develop or contribute to generalizable knowledge."

91. It cannot be reasonably argued that the required research activities under each COVID-19 EUA's Scope of Authorization and the CDC COVID-19 Vaccination Program Provider Agreement do not meet the conditions requiring 45 CFR § 46.101, *et seq*. compliance.

[32] https://www.cdc.gov/coronavirus/2019-ncov/vaccines/safety/pdfs/v-safe-information-sheet-508c.pdf

E. ICCPR Treaty

92. In 1992, the United States Senate ratified the International Covenant on Civil and Political Rights Treaty (ICCPR).[33] Article VII states, "No one shall be subjected to torture or to cruel, inhuman or degrading treatment or punishment. In particular, no one shall be subjected without his free consent to medical or scientific experimentation." (Emphasis added)

93. Subjected means to be under the rule of law by one's authority.

94. Free consent means to be free from outside pressures to participate.

95. The U.S. Senate issued a resolution stating, "That the United States considers itself bound by Article 7 to the extent that 'cruel, inhuman or degrading treatment or punishment 'means the cruel and unusual treatment or punishment prohibited by the Fifth, Eighth and/or Fourteenth Amendments to the Constitution of the United States."[34]

96. The U.S. Senate considered it to be a violation of Article 7 of the ICCPR Treaty and the 5th Amendment's Due Process Clause if individuals were forced to forfeit liberty and property without due process for refusing medical experimentation. The Senate also considered it to be a violation of Article 7 of the ICCPR Treaty and the 14th Amendment's Equal Protection Clause when individuals who refused medical experimentation were treated differently than those who accepted medical experimentation.

[33] Treaty Document 95-20 - INTERNATIONAL COVENANT ON CIVIL AND POLITICAL RIGHTS. (2023, May 19). https://www.congress.gov/treaty-document/95th-congress/20/all-info

[34] See "Resolution" - Treaty Document 95-20 - INTERNATIONAL COVENANT ON CIVIL AND POLITICAL RIGHTS. Congress.gov. Published 2023. Accessed June 5, 2023. https://www.congress.gov/treaty-document/95th-congress/20/all-info

97. The United States Senate stated that Articles One through Twenty-Seven of the ICCPR Treaty are not "self-executing" but "that it is the view of the United States that States Party to the Covenant should, wherever possible, refrain from imposing any restrictions or limitations on the exercise of the rights recognized and protected by the Covenant, even when such restrictions and limitations are permissible under the terms of the Covenant."

98. Treatment by authorities debasing an individual's liberty, autonomy, and human dignity for the express purpose of coercing that individual to surrender their Constitutional rights, leading to feelings of fear, anguish, and inferiority, meets the international definition of cruel, inhumane, and degrading treatment or punishment.[35]

99. Whereas the "United Nations Convention Against Torture and Other Cruel, Inhuman, or Degrading Treatment or Punishment" treaty deals specifically with physical torture or the threat of physical torture, Article VII of the ICCPR Treaty speaks to the political actions of governments and the laws of governments leading to a loss of rights, safety, and liberty, or the feelings that such actions will lead to those losses.

100. The UN Human Rights Committee spoke to Article IV of the ICCPR Treaty regarding the derogation of rights when states declare an emergency. "Article 4, paragraph 2, of the Covenant explicitly prescribes that no derogation from the following articles may be made:

[35]" Treatment that humiliates or debases an individual, showing a lack of respect for, or diminishing, their human dignity, or when it arouses feelings of fear, anguish or inferiority capable of breaking an individual's moral and physical resistance." - degrading treatment or punishment. Published 2023. Accessed June 6, 2023. https://home-affairs.ec.europa.eu/networks/european-migration-network-emn/emn-asylum-and-migration-glossary/glossary/degrading-treatment-or-punishment_en

article 6 (right to life), article 7 (prohibition of torture or cruel, inhuman or degrading punishment, or of medical or scientific experimentation without consent.)"[36] (Emphasis added.)

101. Article 4.2 of the ICCPR Treaty established the restriction of derogation of informed consent rights as a peremptory norm. Although Article VII of the ICCPR Treaty does not provide a private right of action, it is nonetheless enforceable under 42 U.S.C. § 1983 because the treaty contains unambiguous rights enforceable language specific to the individual involved in experimental medical products or processes.

F. 21 U.S.C. §360bbb-3 (aka Section 564)

102. Congress expressly prohibits any manufacturer from introducing into commerce a drug, biologic, or medical device not licensed by the FDA for general commercial marketing (21 U.S.C. §355(a)) to ensure individuals are not subjected to medical experimentation outside of their free consent and or harmed by medical products not effectively researched for safety and efficacy.

103. Investigational drugs, biologics, and devices are strictly controlled by Congress. Only authorized persons may access, distribute, and administer the investigational products and only under the prescribed conditions established by Congress.

104. However, over time, Congress recognized the need to allow individuals to access unlicensed products for emergency, compassionate, and educational purposes (also known as "expanded access protocols"). Therefore, Congress established 21 U.S. Code §360bbb *et seq*., titled "Expanded Access to Unapproved Therapies and Diagnostics."

[36]" No justification or extenuating circumstances may be invoked to excuse a violation of article 7 for any reasons, including those based on an order from a superior officer or public authority." - Human Rights Committee in its General Comment No. 20 on article 7 (A/44/40)

105. Numerous conditions must be met before the legal administration of products authorized pursuant to the section can occur. The overriding requirement, irrespective of the authorized expanded access protocol, is that the individual must give their legally effective informed consent, whether the consent is under written or verbal conditions. This requirement means the authority sponsoring the product or acting on behalf of the sponsor must ensure the individual consenting to participate is under no outside pressure to do so.

106. Making it patently clear of their intent to protect Individuals from medical research abuses, Congress enacted legislation prohibiting federal funding for research activities if the informed consent obtained from the individual is not legally effective nor prospective for the civilian (45 CFR § 46.122) and for the military (10 U.S.C. §980).

107. The Food, Drug, and Cosmetic Act "FDCA"), 21 U.S.C §360bbb-3 authorizes the HHS Secretary to grant emergency expanded access protocols to (1) FDA-licensed products for unlicensed uses or (2) products the FDA has not licensed for general commercial marketing.

108. Congress requires the HHS Secretary to establish "appropriate conditions designed to ensure that individuals to whom the product is administered are informed" of:

> (ii)(II) the significant known and potential benefits and risks of such use, and of the extent to which such benefits and risks are unknown;
>
> (ii)(III) the option to accept or refuse administration of the product;
>
> (ii)(III) the consequences, if any, of refusing administration of the product, and
>
> (ii)(III) the alternatives to the product that are available and of their benefits and risks.[37]

[37] 21 U.S.C. 360bbb-3(e)(1)(A)

109. Informing the individual of the product risks, alternatives, benefits, and health consequences provides that individual with the quality information required to give legally effective informed consent.[38]

110. Congress requires healthcare professionals to inform the individual of "the option to accept or refuse administration of the product," meaning the healthcare professional is required by Congress to inform the individual of his or her legal rights under 21 U.S.C. §360bbb-3 before participating in the product or activity.

111. A legal right is a power held by an individual resulting from a constitution, statute, regulation, or judicial precedent of which no other authority may interfere unless prescribed in law.

112. There are two legal rights conferred upon individuals considering whether to participate in a Section 564 medical countermeasure product, which are (1) the right to **accept** a Section 564 medical product, and (2) the right to **refuse** to take or use a Section 564 medical product.

113. The decision belongs exclusively to the individual, and it must be under conditions free of outside pressures. If individuals are under outside pressure to participate, then it is legally impossible for them to give their free consent; thus, their rights have been infringed upon.

114. There are three specific persons upon whom Congress confers a right under 21 U.S.C. §360bbb-3, which are:

[38] The requirements of informing the subject of risks, benefits, alternatives, and health consequences, and that the Secretary has authorized the use of the investigational drug mirrors 45 CFR §46.116 requirements.

A. the HHS Secretary, who is empowered to authorize access to investigational drugs, biologics, or medical devices and the conditions under which that access can occur,

B. the healthcare professional who is authorized to inform the individual of their Section 564 legal rights and to administer Section 564 medical products, and

C. the individual who is authorized to accept or refuse Section 564 medical products.

115. Congress established a required condition that "[w]ith respect to the emergency use of an unapproved product, the Secretary, to the extent practicable given the applicable circumstances described in subsection (b)(1), shall, for a person who carries out any activity for which the authorization is issued, establish such conditions on an authorization under this section as the Secretary finds necessary or appropriate to protect the public health." (21 USC 360bbb-3(e))

116. Additionally, Congress conferred authority onto the Secretary so that he may "appropriate conditions on who may administer the product with respect to the emergency use of the product, and on the categories of individuals to whom, and the circumstances under which, the product may be administered with respect to such use."

117. These "appropriate conditions" and the "circumstances" are outlined in the Emergency Use Authorization (EUA) letter issued to the manufacturer of the emergency medical countermeasure under the "Scope of Authorization."

118. Therefore, the Scope of Authorization contained in each EUA letter has the force of law as it establishes the conditions under which the emergency activities can occur, prescribing duties for the manufacturer and rights of all persons involved in the administrative process of the product.

119. The Secretary determined that the COVID-19 pandemic required healthcare workers to provide individuals contemplating the use of one of the EUA products with a drug fact sheet *before the product is administered* to act as a function of informed consent. In other words, the Secretary thought it was practical that every person be afforded this right and, as such, mandated that requirement under the Scope of Authorization for each EUA.

120. To ensure individuals are protected when they are offered EUA medical products, Congress was explicit in that "[n]othing in this section [21 U.S.C. 360bbb-3] provides the Secretary any authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section (21 U.S.C. 360bbb-3(l))." (Emphasis added)

121. For purposes of the case at bar, the "activity that becomes lawful pursuant to an authorization under this section" is the administration of the EUA COVID-19 investigational injections manufactured by Pfizer, Moderna, and Johnson & Johnson/Jannsen or the required use of EUA testing articles and masks.

122. Therefore, the Secretary may grant access to unlicensed medical products for use under the declared emergency, but the Secretary may not require any person to manufacture, distribute, store, administer, or receive the product.

123. The Secretary may not delegate his authority, so by extension, any person participating in a 21 U.S.C. §360bbb-3 activity is also restricted by Congress from requiring any other person to participate in "any activity that becomes lawful pursuant to an authorization under" 21 U.S.C. §360bbb-3.

124. **Congress, therefore, prohibits governments (e.g., governors, mayors, school boards) and voluntary participants (e.g., hospitals, manufacturers, doctors) from having the authority to require any person to participate in any 21 U.S.C. §360bbb-3 activity at any time, under any statute, regulation, or state policy or custom.**

125. The explicit purpose of this statutory restriction is to ensure that no person is under a "sanction," "coercion," "undue influence," or "unjustifiable pressure"[39] to participate. If individuals are under those pressures, then no federal funds could be expended for the administration of a COVID-19 EUA product, nor could any healthcare provider acting on behalf of the federal government obtain an individual's Legally Effective Informed Consent.

126. The individual has the right to accept the product, and the healthcare professional has the authority to administer the product. Still, neither is required to act on the demands of the other. Congress established a guideline requiring both the healthcare professional and the individual to mutually agree to the process to meet the legal requirement of 21 U.S.C. §360bbb-3.

127. The purpose of this requirement is to ensure that the individual receives a quality standard of healthcare even under emergency conditions because not everyone is a proper candidate to take or use an investigational medical countermeasure.

128. Therefore, if the HHS Secretary is the only person authorized to establish the conditions under which persons can access 21 U.S.C. §360bbb-3 medical countermeasures and not even he can mandate participation, **then Defendants had no authority to amend the Scope of Authorization and require that which Congress prohibits**.

[39] The Belmont Report's conditions that would nullify legally effective informed consent.

129. Therefore, when Defendants established a policy requiring individuals under their authority to become vaccinated against the COVID-19 virus, they were required by federal law to ensure that (1) licensed products existed to meet the legal requirements of the mandate, and (2) Plaintiffs were to be informed that they were under no obligation to inject an unlicensed COVID-19 EUA investigational drug into their body, nor would they incur a penalty or lose a benefit when refusing to do so. Defendants did neither.

G. HHS EUA Precedent

130. On January 28, 2005, HHS issued the first EUA[40] under its new Section 564 authority (i.e., 21 USC 360bbb-3). The military requested EUA protocols for Anthrax Vaccine Adsorbed (AVA), to be utilized by civilians and service members. HHS stated, "The issuance of this Authorization for the emergency use of AVA is the first time that the EUA authority is being used. FDA intends to explain clearly the reasons for each issuance, termination, or revocation of an EUA. The agency wishes to make its decision-making understandable to help ensure that members of the public, and particularly those individuals who may be eligible to receive a medical product authorized for emergency use, are informed about the basis of an EUA determination."

131. HHS mandated that individuals participating in the AVA investigational product must be informed of the following statements:

> A. Individuals (service members and civilians) who refuse anthrax vaccination will not be punished. (Emphasis added)
>
> B. Refusal may not be grounds for any disciplinary action under the Uniform Code of Military Justice.
>
> C. Refusal may not be grounds for any adverse personnel action. Nor would either military or civilian personnel be considered non-

[40] https://www.govinfo.gov/content/pkg/FR-2005-02-02/pdf/05-2028.pdf

deployable or processed for separation based on refusal of anthrax vaccination.

D. There may be no penalty or loss of entitlement for refusing anthrax vaccination,

E. This information shall read in the trifold brochure provided to potential vaccine recipients as follows: You may refuse anthrax vaccination under the EUA, and you will not be punished. No disciplinary action or adverse personnel action will be taken. You will not be processed for separation, and you will still be deployable. There will be no penalty or loss of entitlement for refusing anthrax vaccination.[41]

132. The explicit instructions in the EUA language directly relate to AVA's classification as an investigational new drug not licensed by the FDA for any legal indication. Moreover, the language was designed to ensure that healthcare professionals could obtain the legally effective informed consent of the individual because it expressly informed the individual that no "sanction" would be imputed for refusal, thus nullifying all outside pressures to participate. No amendments to Section 564 have altered its requirements since HHS issued this first EUA.

133. The reason HHS was crystal clear about an individual's right to refuse an investigational drug was to respect court orders and the express authority of individuals to choose the available statutory options.

H. Judicial EUA Precedent

134. On October 27, 2004, U.S. District Court Judge Sullivan spoke to the individual's authority to refuse investigational drugs without consequence when he held in *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004), that:

[41] Federal Register/Vol. 70, No. 21/Wednesday, February 2, 2005/Notices 5455 IV Conditions of Authorization

> A. Congress has prohibited the administration of investigational drugs to service members without their consent. This Court will not permit the government to circumvent this requirement; and,
>
> B. Unless and until FDA properly classifies AVA [an anthrax vaccine] as a safe and effective drug for its intended use, an injunction shall remain in effect prohibiting defendants 'use of AVA on the basis that the vaccine is either a drug unapproved for its intended use or an investigational new drug within the meaning of 10 U.S.C. §1107. Accordingly, the involuntary anthrax vaccination program, as applied to all persons, is rendered illegal absent informed consent or a Presidential waiver." (Emphasis added.)

135. Immediately upon Judge Sullivan's ruling, the Department of Defense ended all punitive activities against service members and civilian employees because the federal court affirmed the individual's statutory authority to refuse investigational drugs without consequence. Except for 10 U.S.C. § 1107, the laws leading Judge Sullivan to his ruling apply to individuals irrespective of civilian or military service. No laws have changed to negate Judge Sullivan's 2004 ruling.

136. Judge Sullivan added clarity to the importance of what was argued before the court by stating: "The Court is persuaded that the right to bodily integrity and the importance of complying with legal requirements, even in the face of requirements that may potentially be inconvenient or burdensome, are among the highest public policy concerns one could articulate." *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004).

137. *Doe* and the HHS provide judicial and administrative precedent affirming the right of individuals to refuse investigational products without incurring a penalty or losing a benefit to which they are otherwise entitled. Nothing in the law has changed to nullify that right since those precedents were firmly established.

I. Federal Wide Assurance (FWA)

138. In 2001, HHS created the Office of Human Rights Protection (OHRP), which established the Federal Wide Assurance (FWA) agreement. The FWA is an agreement by entities conducting business with HHS to comply with 45 CFR 46 and the Belmont Report's ethical guidelines.

139. HHS states, "The Federal Wide Assurance (FWA) is an assurance of compliance with the U.S. federal regulations for the protection of human subjects in research. It is approved by the Office for Human Research Protections (OHRP) for all human subjects research conducted or supported by the U.S. Department of Health and Human Services (HHS). The FWA is also approved by OHRP for federal wide use, which means that other U.S. federal departments and agencies that have adopted the U.S. Federal Policy for the Protection of Human Subjects (also known as the Common Rule) may rely upon the FWA for the research they conduct or support. An FWA is the only type of assurance currently accepted and approved by OHRP. It is required whenever an Institution becomes engaged in human subjects research conducted or supported by any U.S. federal department or agency that has adopted the Common Rule…"[42]

140. The OHRP assigns an FWA identification number to entities (hereinafter referred to as "Contracting Provider") that fulfill application requirements. An FWA identification number is issued only after the legally binding agreement between the Contracting Provider and the United States government has been signed.

141. The FWA's main purpose is to benefit a third-party beneficiary because the FWA agreement authorizes the Contracting Provider to participate in federally funded programs

[42] Office for Human Research Protections. Federal Wide Assurance Instructions. HHS.gov. Published January 7, 2011. Last accessed May 19, 2023.

involving humans with investigational drugs if, and only if, the Contracting Provider agrees to protect the health and legal rights of the third-party beneficiaries (i.e., humans who are administered investigational drugs, biologics, or devices under the research conditions described above).

142. The fact that the entire FWA agreement hinges upon the intended rights of third-party beneficiaries means that Contracting Providers have a fiduciary duty to the third-party beneficiaries under the terms of the FWA agreement.

143. The intended benefit to the third-party beneficiary is the right to accept or refuse participation in investigational products, clinical trials, and other research activities without fearing consequences for refusal and to know that independent Institutional Review Boards will provide oversight, ensuring their health, safety, and rights are protected.

144. Although the third-party beneficiaries are not signatories to the contract, they are the intended third-party beneficiaries of the agreement, and their rights were violated the moment Defendants penalized Plaintiffs for refusing to take EUA products (i.e., investigational drugs, and testing articles).

145. The FWA agreement requires the Contracting Provider to ensure that no third-party beneficiary is under outside pressure to participate in an investigational drug, biologic, or medical device.

146. The FWA agreement requires Defendants to assure potential participants that they will not incur a penalty or lose a benefit to which they are otherwise entitled when refusing participation.[43]

[43]" The Federal Wide Assurance (FWA) is the only type of assurance currently accepted and approved by OHRP. Through the FWA, an institution commits to HHS that it will comply with the requirements in the HHS Protection of Human Subjects regulations at 45 CFR part 46." - HHS. 45 CFR 46.116(b)(8) requires the individual to be informed they will not be penalized for refusing participation in a research activity.

147. The duty placed upon the Contracting Provider is owed to those who refuse as well as those who accept the administration of investigational drugs.

148. Therefore, when Defendants punished Plaintiffs (third-party beneficiaries) for refusing the administration of an investigational drug, they:

A. activated the terms and conditions of the contract,

B. violated the terms of the contract, causing injury to the legal rights of the third-party beneficiary,

C. created a cause of action for breach of fiduciary duty in favor of the third-party beneficiary.

149. The Fourteenth Amendment's Equal Protection Clause provides additional protections by requiring all persons involved in federally funded COVID-19 countermeasure programs to be treated equally before the law, irrespective of the chosen option.

J. Preemption of State Law – PREP Act and EUA Statute

150. In 2005, Congress passed the Public Readiness and Emergency Preparedness Act, hereafter referred to as the PREP Act (42 USC 247d-6d and 42 USC 247d-6e), to provide immunities for persons volunteering for "covered" activities. In accordance therewith, the HHS Secretary issued a COVID-19 PREP Act declaration in February 2020.[44]

151. The first provision of the PREP Act (42 USC 247d-6d) is entitled "Targeted liability protections for pandemic and epidemic products and security countermeasures."

152. The second provision of the PREP Act (42 USC 247d-6e) is entitled "Covered countermeasure process."

153. Congress expressly crafted language preempting state and local law conflicting with the PREP Act (42 USC 247d-6d(b)(8)), which provides, in pertinent part:

[44] 85 FR 15198

(8) Preemption of State law

> During the effective period of a declaration under subsection (b)…no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that—
>
> (A) is different from, or is in conflict with, any requirement applicable under this section; and
>
> (B) relates to the…administration…of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 et seq.].

154. Congress expressly established that participation in the administration of the covered countermeasure (i.e., any EUA COVID-19 investigational drug) shall be voluntary. Specifically, Congress stated the following at 42 USC 247d-6e(c), in pertinent part:

(c) Voluntary program

> The Secretary shall ensure that a State, local, or Department of Health and Human Services plan to administer or use a covered countermeasure is consistent with any declaration under 247d–6d of this title…and that potential participants are educated with respect to contraindications, the voluntary nature of the program, and the availability of potential benefits and compensation under this part. [Emphasis added.]

155. The CDC COVID-19 Vaccination Provider Program exclusively utilizes "the relevant state, local, or territorial immunization's cooperative agreement with CDC" as the means to distribute the federal government's "covered countermeasure[s]" (COVID-19 EUA drugs) (see, *infra*).

156. Therefore, when the State voluntarily agreed to use its immunization program to distribute the federal government's property, it was required to ensure all participants were only involved under strictly voluntary conditions.

157. Congress informed the State that if it planned to "administer or use a covered countermeasure," then it may not "establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that…is different from, or is in conflict with…**any matter included in a requirement** applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act."

158. One such **matter included in a requirement applicable to the covered countermeasure** is contained in 21 U.S.C. §360bbb-3 (the Emergency Use Authorization statute) whereby Congress, in unambiguous rights-conferring language, conferred upon the individual considering participation in a "covered countermeasure" may choose to "accept" or "refuse" participation.[45]

159. Therefore, by the express language of the PREP Act, and its incorporation of the option to choose contained in the EUA statute, and also based on the Supremacy Clause in the U.S. Constitution, any State laws, ordinances, regulations, or customs that are "different" from or in "conflict" with an individual's authority to freely choose to accept or refuse participation in the medical countermeasure are preempted.

160. Therefore, States are preempted by Congress from mandating participation in a PREP Act "covered countermeasure."

161. Similarly, the State and private employers in that State are prohibited from enforcing any at-will employment doctrine when employees refuse participation in a "covered countermeasure" because using the threat of penalty of loss of employment benefits, or even employment itself, "conflict[s]" with the employee's federally secured right to accept or refuse

[45] 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III)

participation in the covered countermeasure (e.g., drugs, biologics, masks, testing articles, etc.) without consequence.[46]

162. The purpose of informing the individual of "the significant known…risks of such use, and of the extent to which such benefits and risks"[47] and of "the alternatives to the product that is available and of their benefits and risks"[48] is because the individual is not only consenting to be *irreparably* injected with an investigational drug but to also participate in a legally binding agreement under the terms and conditions established by Congress.

163. Individuals who consent to receive one of the COVID-19 EUA investigational drugs must agree to the following terms and conditions, including but not limited to:

A. forfeiture of civil litigation rights resulting from injuries;[49]

B. allowing their private identifiable information to be collected and used for a variety of purposes by unknown persons;[50]

C. allowing their involvement with the EUA product to be cataloged by various persons for unknown purposes,

D. allowing the data collected about their adverse events to be utilized by researchers for unknown purposes and eternity,[51]

E. assuming greater risks to their safety, health, and legal rights.[52]

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[46] *Id.*

[47] (21 U.S.C. §360bbb-3(e)(1)(A)(ii)(II))

[48] (21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III))

[49] PREP Act forfeits all civil actions for damages in most situations.

[50] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Agreement requires manufacturers and/or emergency stakeholders to obtain private identifiable information.

[51] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Agreement requires manufacturers and/or emergency stakeholders to monitor, report and study a variety of adverse reactions to EUA products.

[52] Section 564 (21 U.S.C. 360bbb-3) requires potential recipients to be made aware of the risks, alternatives, and the fact that the product is only authorized by the Secretary under emergency conditions. These elements provide potential recipients with the required information to make a quality and legally effective decision to consent. Therefore, consent means the individual agrees to assume more than minimal risk as defined above.

164. The FDA issued an opinion[53] regarding federal preemption of the State's authority to interfere with 21 U.S.C. §360bbb-3 (aka section 564):

> FDA anticipates that conflicts between federal and state law may arise when FDA acts under sections 564, 564A, and 564B if states have existing requirements governing the shipment, holding, dispensing, administration, or labeling of unapproved medical products or approved medical products for unapproved uses. Courts have stated that the Supremacy Clause of the U.S. Constitution can operate to nullify both state legislative requirements and state common-law duties. Under the legal principles of implied conflict preemption, courts have found state law preempted where it is impossible to comply with both federal and state law, or when the state law "stands as an obstacle to the accomplishment and execution of the full purposes and objectives of Congress." Consistent with this case law, section 4(a) of Executive Order 13132 states that "[a]gencies shall construe... a Federal statute to preempt State law only where the statute contains an express preemption provision or there is some other clear evidence that the Congress intended preemption of State law, or where the exercise of State authority conflicts with the exercise of Federal authority under the Federal statute." FDA states that the terms and conditions of an EUA issued under section 564 preempt state or local law, both legislative requirements and common-law duties, that impose different or additional requirements on the medical product for which the EUA was issued in the context of the emergency declared under section 564. Similarly, an order or waiver issued under section 564A and pre-positioning under section 564B preempt state or local law, both legislative requirements and common-law duties, that impose different or additional requirements related to the activity authorized under sections 564A or 564B. To the extent state or local law may impose requirements different from or in addition to those imposed by the EUA for a particular medical product within the scope of the declared emergency or threat of emergency (e.g., requirements on prescribing, dispensing, administering, or labeling of the medical product), such law "stands as an obstacle to the accomplishment and execution of the full purposes and objectives of Congress," and "conflicts with the exercise of Federal authority under [§ 564]." The same rationale applies to an order or waiver issued under section 564A and pre-positioning of an MCM under section 564B. (Emphasis added)

[53] "Emergency Use Authorization of Medical Products and Related Authorities," Section VII. U.S. Food and Drug Administration. Published 2022. Accessed June 6, 2023. https://www.fda.gov/regulatory-information/search-fda-guidance-documents/emergency-use-authorization-medical-products-and-related-authorities#preemption

165. The Supreme Court has long held that "the test of applicability of state laws [conflicting with the Supremacy Clause] is whether the matter on which the State asserts the right to act is in any way regulated by the Federal Act."[54]

166. 21 U.S.C. §360bbb-3 and the PREP Act can only be executed by the United States HHS Secretary under the prescribed conditions established by Congress.

167. The State and subordinate private parties may only participate in "covered countermeasures" and the use of 21 U.S.C. §360bbb-3 medical products by volunteering to adhere to the conditions established by the HHS Secretary under the respective statutes, which includes the federal statutory requirement to obtain Plaintiffs' legally effective informed consent when considering participation in programs authorized by the above statutes.

168. If Defendants can punish Plaintiffs for *refusing* to participate in using a 21 U.S.C. §360bbb-3 product, they must also have the authority to punish Plaintiffs *accepting* the product. In such a scenario, one can easily see that Plaintiffs are damned if they accept and damned if they refuse because the option no longer belongs to the Plaintiffs; rather, it belongs to Defendants disagreeing with the Plaintiffs' choice. Granting Defendants that power deprives Plaintiffs of their Constitutional rights (Equal Protection of Laws and Due Process) and undermines the authority of Congress to determine the conditions under which access to unlicensed drugs, biologics, and devices can occur.

169. Moreover, should Defendants be allowed to interfere in the Federal Acts by penalizing Plaintiffs for refusing to participate, and Plaintiffs are injured, having no judicial recourse for remedy, then Plaintiffs are deprived of their Due Process rights resulting from the sustained losses of their injury. Should a person be informed of the risks of participating in a

[54] *Rice v. Santa Fe Elevator Corp.*, 331 U.S. 218, 236 (1947)

covered countermeasure and still choose to participate, resulting in injury, then courts are satisfied that their Due Process rights are not violated when denied judicial relief under the statute's immunity clauses because they were made aware of the risks and consequences prospectively.

170. This irrefutable fact is why Congress preempts the State and Defendants (as State Actors) from having any authority to interfere with the right of Plaintiffs to either accept or refuse participation in "medical countermeasures" under 21 U.S.C. §360bbb-3 and "covered countermeasures" under the PREP Act.

171. In the case at bar, Defendants' use of the state at-will employment doctrine to terminate Plaintiffs' employment as a penalty for refusing administration of an EUA investigational drug conflicts with the federal law's goal of ensuring that only truly willing participants are involved in the use of "covered countermeasures" and 21 U.S.C. §360bbb-3 medical products, and as a result of that conflict, the State's at-will employment doctrine is preempted for purposes of the administration of those countermeasures. It is thereby inapplicable as a defense to Defendants' unlawful actions described herein by Plaintiffs.

K. CDC COVID-19 Vaccination Program Provider Agreement

172. The Centers for Disease Control (CDC) states that "[a]t this time, **all COVID-19 vaccine in the United States has been purchased by the U.S. government** (USG) for administration exclusively by providers enrolled in the CDC COVID-19 Vaccination Program and remains U.S. government property until administered to the recipient. Only healthcare professionals enrolled through a health practice or organization as vaccination providers in the CDC COVID-19 Vaccination Program (and authorized entities engaged in shipment for the Program) are authorized to lawfully possess, distribute, deliver, administer, receive shipments of, or use USG-purchased COVID-19 vaccine. Other possession, distribution, delivery,

administration, shipment receipt, or use of COVID-19 vaccine outside the parameters of the Program constitutes, at a minimum, theft under 18 U.S.C. § 641, and violation of other federal civil and criminal laws. Violators are subject to prosecution to the full extent of the law." [See Exhibit A, Provider Agreement]

173. Although the program states it is a "Vaccination Program" (hereinafter referred to as "CDC Vaccination Program"), the federal government has not distributed any FDA-licensed COVID-19 vaccines. Instead, it has relied exclusively on unlicensed COVID-19 EUA drugs for the program's administration.

174. Before the CDC accepts a person or entity as a Provider in the CDC Vaccination Program, that person or entity is required to sign the CDC COVID-19 Vaccination Program Provider Agreement (See Exhibit A, Provider Agreement).

175. The Provider Agreement informs the person or entity that, "Your Organization's chief medical officer (or equivalent) and chief executive officer (or chief fiduciary)—collectively, Responsible Officers—must complete and sign the CDC COVID-19 Vaccination Program Provider Requirements and Legal Agreement (Section A)" (See Exhibit A, Provider Agreement.)

176. The Provider Agreement requires the organization to assign a person or persons who will be under a legal obligation to ensure the program is carried out effectively, declaring, "For the purposes of this agreement, in addition to Organization, Responsible Officers named below will also be accountable for compliance with the conditions specified in this agreement. The individuals listed below must provide their signature after reviewing the agreement requirements."

177. "This program is a part of collaboration under the relevant state, local, or territorial immunization's cooperative agreement with CDC. To receive one or more of the publicly funded

COVID-19 vaccines (COVID-19 Vaccine), constituent products, and ancillary supplies at no cost, Organization agrees that it will adhere to the following requirements…" (Emphasis added).

178. Therefore, the CDC clearly states that the Provider Agreement works in conjunction with "relevant state" and other municipality immunization agreements. This requirement denotes state action involving private parties acting in the capacity of a state actor.

179. HHS requires any entity conducting business with its organization to submit and be approved for a Federal Wide Assurance agreement (see discussion, *supra*) in advance of participating in any program involving humans with investigational drugs under its authority. The fact that the medical products in question are under an EUA does not exempt entities conducting business with HHS from first agreeing to obtain an FWA before participation. This fact is why the CDC chose only to distribute the program via the State's existing immunization program. Each state already has an HHS FWA agreement in place.

180. The Executive Branch of the government is required to comply with 21 U.S.C. §360bbb-3's requirements and all other laws and regulations protecting humans involved in investigational medical products under emergency access protocols.

181. When the Executive Branch chose to purchase all COVID-19 vaccines (i.e., licensed and unlicensed COVID-19 drugs), it was required to ensure that all applicable laws associated with each drug's classification were adhered to by all volunteering participants.

182. The Executive Branch chose to establish the Provider Agreement as the mechanism to ensure those legal obligations were followed.

183. While the Provider Agreement does not replace the laws and regulations governing any EUA drug classification, it adds an extra layer of legal obligations required of volunteer participants.

184. The Provider Agreement requires that all volunteer participants:

A. "must provide an approved Emergency Use Authorization (EUA) fact sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative,"

B. "Organization must report moderate and severe adverse events following vaccination to the Vaccine Adverse Event Reporting System (VAERS),"

C. "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine,"

D. "Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws."

185. The EUA Fact Sheet is required because the Executive Branch of the government is the sole sponsor of EUA products,[55] and federal law requires them to obtain the legally effective informed consent of each individual before the administration of the product. Moreover, the HHS Secretary requires each recipient to be given the Fact Sheet for each EUA COVID-19 investigational drug from which the federal branch of government cannot exempt itself. The required Fact Sheet acts as a function of the "informed consent" process for persons ascertaining whether or not they will participate in the EUA product.

186. The Executive Branch is required to report adverse events as part of the government's COVID-19 Vaccination Program because federal law requires this of every EUA product, which the HHS Secretary echoed in each of the EUA letters issued to pharmaceutical companies. Moreover, the requirement to monitor, collect, and report, adverse reactions (research

[55] The Federal government chose to purchase and retain ownership of all EUA COVID-19 drugs. However, that ownership does not negate their legal obligations under Section 564.

activities) from the drugs 'use denotes how these products are governed by 45 CFR 46, requiring both IRB and Belmont Report compliance.

187. The requirement that the "Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws" is because federal law declares:

A. "This policy does not affect any state or local laws or regulations (including tribal law passed by the official governing body of an American Indian or Alaska Native tribe) that may otherwise be applicable and that provide additional protections for human subjects" (45 CFR 46.101(f));

B. Additionally, federal law declares, "The informed consent requirements in this policy are not intended to preempt any applicable Federal, state, or local laws (including tribal laws passed by the official governing body of an American Indian or Alaska Native tribe) that require additional information to be disclosed in order for informed consent to be legally effective" (45 CFR 46.116(i));

C. This policy does not affect any foreign laws or regulations that may otherwise be applicable and that provide additional protections to human subjects of research (45 CFR 46.101(g)).

188. The Provider Agreement required Defendants to acknowledge the law before acceptance, as follows: "By signing this form, I certify that all relevant officers, directors, employees, and agents of Organization involved in handling COVID-19 Vaccine understand and will comply with the agreement requirements listed above…" (Emphasis added)

189. Therefore, State governments and their authorized private parties agreed to participate in a joint effort to conduct research activities and obtain the legally effective informed consent of individuals on behalf of the United States Government when signing the CDC COVID-19 Vaccination Program Provider Agreement.

VI. Statement Of Facts

190. Plaintiffs make no assertions regarding whether it is lawful for a public or private entity to mandate taking a ***licensed*** vaccine. Plaintiffs 'allegations herein only relate to Defendants 'mandating the use of drugs, biologics, and devices authorized under 21 U.S.C. §360bbb-3 and the PREP Act.

191. Plaintiffs adamantly assert that an individual has the absolute Constitutional and federally secured right to refuse the administration of an Emergency Use Authorization (EUA) drug (e.g., Pfizer BioNTech COVID-19 Vaccine), biologic, or device (e.g., EUA testing articles and masks) without incurring a penalty or losing a benefit to which they are otherwise entitled. Moreover, such a right is not dependent upon a person seeking a religious or medical exemption.

192. Plaintiffs assert that they have the Constitutional and federal statutory right to refuse the administration of an EUA product and refuse participation in any activity or product covered by the PREP Act.

193. Plaintiffs assert that Defendants are prohibited by Congress from establishing conditions requiring Plaintiffs to surrender their statutory rights and Constitutional protections as a condition to participate in privileges and benefits offered under federal and Washington State laws, regulations, ordinances, and customs.

194. Plaintiffs were employed as healthcare workers licensed by the State of Washington, working in healthcare facilities also licensed by the State, which has the authority to deny healthcare facilities and workers the right to conduct commerce by revoking their respective State licenses.

195. In August 2021, Governor Inslee, acting under color of law, issued official proclamations 21-14 and 21-14.1[56] **prohibiting**:

A. "Any Health Care Provider from failing to be fully vaccinated against COVID-19 after October 18, 2021;" and,

B. "Any operator of a Health Care Setting from permitting a Health Care Provider to engage in work for the operator as an employee, contractor, or volunteer in their capacity as a Health Care Provider after October 18, 2021 if the Health Care Provider has not been fully vaccinated against COVID-19 and provided proof thereof as required below. Providers who do not work in a Health Care Setting must provide proof of vaccination to the operator of the facility in which the Provider works, if any, or, if requested, to a lawful authority. A lawful authority includes, but is not limited to, law enforcement, local health jurisdictions, and the state Department of Health."

196. Proclamation 21-14.1 defines fully vaccinated as:

A. "A person is fully vaccinated against COVID-19 two weeks after they have received the second dose in a two-dose series of a COVID-19 vaccine (e.g., Pfizer-BioNTech or Moderna) or a single-dose COVID-19 vaccine (e.g., Johnson & Johnson (J&J)/Janssen) authorized for emergency use, licensed, or otherwise approved by the FDA or listed for emergency use or otherwise approved by the World Health Organization."

197. From the date Governor Inslee issued his mandatory vaccination requirements until he rescinded that requirement, no FDA-licensed COVID-19 vaccine had been introduced into commerce for general commercial marketing. Thus, no FDA-licensed COVID-19 vaccine was available for healthcare workers to use to comply with Gov. Inslee's proclamation. Therefore, Governor Inslee exclusively relied on investigational new drugs under 21 U.S.C. §360bbb-3 authorization for Plaintiffs to comply with his 21-14-1 Proclamation.

[56] See Exhibit C - Proclamation 21-14-1

198. No person has the legal authority to require another person to inject an unlicensed EUA investigational drug into their body as a condition to earn, receive, or enjoy a privilege of the State or conduct commerce, including employment. (See, discussion *infra*)

199. On December 11, 2020, the FDA issued to Pfizer-BioNTech the first COVID-19 EUA for its investigational drug (officially named Pfizer-BioNTech COVID-19 Vaccine[57]), and the FDA confirmed that Pfizer's product "is an investigational vaccine not licensed for any indication."[58]

200. On December 18, 2020, the FDA issued to ModernaTX, Inc., an EUA for its investigational drug (officially named Moderna COVID-19 Vaccine), and the FDA confirmed that Moderna's product "is an investigational vaccine not licensed for any indication."[59]

201. On February 27, 2021, the FDA issued to Janssen Biotech, Inc., an EUA for its investigational drug (officially named Janssen COVID-19 Vaccine), and the FDA confirmed that Janssen's product "is an investigational vaccine not licensed for any indication."[60]

202. Investigational new drugs[61] (IND) have no FDA-licensed legal indication to treat, cure, or prevent any known disease and are experimental by their very nature.[62]

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[57] The FDA improperly allowed Pfizer to add the word "Vaccine" to its investigational name. The court should not confuse this name to mean the drug is legally indicated for use as a vaccine. Pfizer-BioNTech COVID-19 Vaccine is an investigational drug having no legal indication to treat, cure, or prevent any known disease. The FDA classified the drug as an "investigational new drug." See Exhibit D, FDA's EUA Scope of Authorization Letter to Pfizer, December 11, 2020.

[58] 86 Fed.Reg. 5200, Jan. 19, 2021

[59] 86 Fed.Reg. 5211, Jan. 19, 2021

[60] 86 Fed.Reg. 28608, May 27, 2021

[61] Investigational drug "means a new drug or biological drug that is used in a clinical investigation." (21 CFR 312.3 "Investigational new drug") Clinical investigation "means any experiment in which a drug is administered or dispensed to, or used involving, one or more human subjects. For the purposes of this part, an experiment is any use of a drug except for the use of a marketed drug in the course of medical practice." (21 CFR 312.3 "Clinical investigation") (Emphasis added).

[62] Investigational new drug means, "A substance that has been tested in the laboratory and has been approved by the U.S. Food and Drug Administration (FDA) for testing in people…Also called experimental drug, IND, investigational

203. Therefore, it is indisputable that Governor Inslee deprived Plaintiffs of their Constitutional and federal statutory rights to refuse an EUA/PREP Act product when he required Washington healthcare workers to inject an unlicensed investigational new drug into their bodies before October 18, 2021, as a condition to continue employment in their chosen healthcare profession.

204. Governor Inslee stated in his Proclamation 21-14-1, "Where required above, Workers for State Agencies, Workers for operators of Educational Settings, and Health Care Providers must provide proof of full vaccination against COVID-19."

205. Factually, Plaintiffs would never have been able to provide proof of full vaccination because no FDA-licensed vaccines existed to vaccinate persons from any SARS-CoV-2 (COVID-19) variant.

206. Drugs and biologics are regulated according to their classification and not their formulation, and the EUA classification has no legal indication as a "vaccine." That is why the FDA stated in its respective EUA letters to the manufacturers that the drugs were "investigational vaccines not licensed for any indication," meaning they were under investigation to receive the vaccine classification potentially.

207. Governor Jay Inslee willfully enrolled Washington State in the CDC COVID-19 Vaccination Program. The CDC informed Governor Inslee that,

> "[a]t this time, **all COVID-19 vaccine in the United States has been purchased by the U.S. government** (USG) for administration exclusively by providers enrolled in the CDC COVID-19 Vaccination Program and remains U.S. government property until administered to the recipient. Only healthcare professionals enrolled through a health practice or organization as vaccination providers in the CDC COVID-19 Vaccination Program (and

agent, and investigational drug." NCI Dictionary of Cancer Terms. National Cancer Institute. Published 2023. Accessed June 25, 2023. https://www.cancer.gov/publications/dictionaries/cancer-terms/def/investigational-new-drug

> authorized entities engaged in shipment for the Program) are authorized to lawfully possess, distribute, deliver, administer, receive shipments of, or use USG-purchased COVID-19 vaccine. Other possession, distribution, delivery, administration, shipment receipt, or use of COVID-19 vaccine **outside the parameters of the Program** constitutes, at a minimum, theft under 18 U.S.C. § 641, and violation of other federal civil and criminal laws. Violators are subject to prosecution to the full extent of the law." (Emphasis added) [See Exhibit A – CDC Covid-19 Vaccination Program Provider Agreement ("Provider Agreement").]

208. The Provider Agreement is an agreement among the federal government, the state governments, and the healthcare providers who contractually agree to administer the injections to the public in accordance with the terms and conditions of the Provider Agreement and the federal statutes referenced therein.

209. The Provider Agreement states, "This program is a part of **collaboration** under the relevant state, local, or territorial immunization's cooperative agreement with CDC. To receive one or more of the publicly funded COVID-19 vaccines (COVID-19 Vaccine), constituent[63] products, and ancillary supplies at no cost, Organization agrees that it will adhere to the following requirements…" (emphasis added).

210. The "Organization" refers to the healthcare facility signing the Provider Agreement and agreeing to administrate the federal government's property. The "Organization" is either a state health clinic, health care professional licensed by the State, healthcare facility licensed by the State, or other person authorized by the State to administer the federal property on the State's behalf.

211. The CDC informed Governor Inslee and Shriners PolicyMakers that "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug

[63] 21 CFR 312. §4.2 "constituent…is part of a combination product" defined in 21 CFR § 3.2(e).

Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine."[64] This requirement not only applied to Washington State and its various health agencies and clinics, but it also applied to all private healthcare facilities (and their employees) licensed by the State of Washington and signed the Provider Agreement.

212. Therefore, since the COVID-19 drugs available to persons under Governor Inslee's mandate were only authorized under the EUA statute, Governor Inslee was legally bound to ensure that his mandate complied with "all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA[65] that covers COVID-19 Vaccine."

213. 21 U.S.C. §355(a) states that "no person shall introduce or deliver for introduction into interstate commerce any new drug unless an approval of an [FDA marketing] application." (Emphasis added).[66]

214. Congress carved out exemptions to that restriction under the EUA statute to allow individuals access to unlicensed drugs and biologics under compassionate, educational, and emergency conditions.[67]

215. Congress established **a required condition** before expanded access protocols could be issued under 21 U.S.C. §360bbb-3, "[w]ith respect to the emergency use of an unapproved product, the Secretary, to the extent practicable given the applicable circumstances described in subsection (b)(1), **shall**, for a person who carries out any activity for which the authorization is

[64] See 12(a) in the Provider Agreement.
[65] 21 U.S.C. §360bbb-3
[66] 21 U.S.C. §355 (New Drugs) is part of Chapter 9 – Federal Food, Drug, and Cosmetic Act, Subchapter VI – Drugs and Devices, Part A – Drugs and Devices.
[67] The EUA statute is also part of Chapter 9 – Federal Food, Drug, and Cosmetic Act.

issued, **establish such conditions** on an authorization under this section as the Secretary finds necessary or appropriate to protect the public health." (21 USC 360bbb-3(e)) (emphasis added)

216. Congress also conferred authority onto the Secretary that he may "appropriate conditions on who may administer the product with respect to the emergency use of the product, and on the categories of individuals to whom, **and the circumstances under which**, the product may be administered with respect to such use." (21 U.S.C. §360bbb-3(e)(1)(B)(ii)) (Emphasis added).

217. However, because for many years prior to 2021 it was clearly established that COVID-19 drug EUAs must comply with 45 CFR 46[68] and the Belmont Report, Governor Inslee was aware that "[n]othing in this section provides the Secretary **any authority to require any person to carry out any activity** that becomes lawful pursuant to an authorization under this section (21 U.S.C. 360bbb-3(l))" (Emphasis added). In other words, not even the Secretary of HHS may require any person to manufacture, distribute, administer, or receive an EUA product, but Gov. Inslee attempted to do so.

218. Governor Inslee, acting with moral turpitude,[69] removed the Constitutional and federal statutory right of healthcare workers to refuse the administration of an unlicensed investigational drug. Ironically, the Governor ordered healthcare professionals to act in a manner that was not professional and to forgo their oath to do no harm by ignoring potentially trillions of

[68] 45 CFR §46.122: "Federal funds administered by a Federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied." "Research means a systematic investigation, including research development, testing, and evaluation, designed to develop or contribute to generalizable knowledge. Activities that meet this definition constitute research for purposes of this policy, whether or not they are conducted or supported under a program that is considered research for other purposes." - 45 CFR §46.102(1) (emphasis added)

[69] "This phrase is used to describe the violation of decent, moral, and honest behavior and an act of depravity or vileness." Black's Law Dictionary 2nd Ed.

contraindications of the three mRNA investigational new drugs available when he issued his edict (see discussion, *infra*).

219. Therefore, Congress provided only the HHS Secretary authority to grant expanded access protocols to unlicensed medical products undergoing clinical trials or products not licensed for their intended use under a declared emergency. However, Congress restricted the Secretary from requiring anyone to manufacture, distribute, administer, or receive the product simply because the product was granted emergency access. Moreover, Congress did not authorize the HHS Secretary to delegate his authority to another person. By extension, any person involved in a 21 U.S.C. §360bbb-3 activity is also restricted from requiring any person to participate in that activity.

220. The moment Governor Inslee volunteered the State of Washington to participate in the CDC COVID-19 Vaccination Provider Program, he agreed to the terms and conditions of the program, which required him to comply with all protocols under the EUA statute, including the restriction that no person can be required to participate in any EUA activity. However, even if no Provider Agreement existed, Governor Inslee still must abide by the EUA statute anytime the State involves an individual with an EUA investigational drug. Therefore, when he issued his mandate, Gov. Inslee violated statutory and contractual duties and deprived Plaintiffs of their Constitutional and federally secured rights because his mandate relied exclusively on EUA and PREP Act products for compliance.

221. Specifically, when Governor Inslee issued an Executive Proclamation prohibiting "Any operator of a Health Care Setting from permitting a Health Care Provider to engage in work for the operator as an employee, contractor, or volunteer in their capacity as a Health Care Provider after October 18, 2021 if the Health Care Provider has not been fully vaccinated against COVID-

19" he fraudulently misrepresented his authority to Plaintiffs and established EUA protocols that Congress legally prohibited.

222. Moreover, Governor Inslee freely volunteered to comply with the CDC COVID-19 Vaccination Program Provider Agreement, which contains third-party beneficiary rights for Plaintiffs. Yet, he violated those rights and deprived Plaintiffs of their Constitutional and federal statutory rights.

223. Governor Inslee, at no time, from the beginning of the pandemic through the filing of this Complaint, provided Plaintiffs with information about their legal right to either accept or refuse EUA products without consequence. Governor Inslee intentionally refrained from referencing 21 U.S.C. §360bbb-3 in any of his Proclamations because that statute proves his mandates were unlawful because he lacked authority to issue such a proclamation and the requirement to inject unlicensed drugs into Plaintiffs 'bodies infringes upon the authority of Plaintiffs to freely choose their preferred "option" under 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III), among other Constitutional rights.[70]

224. The Supremacy Clause doctrine establishes that 21 U.S.C. §360bbb-3 supersedes Washington State's and Governor Inslee's authority to establish conditions conflicting with the federal statute. Because Congress restricted the HHS Secretary from delegating his authority, Governor Inslee unlawfully enacted legislation outside of the authority of the United States Congress, creating a Constitutional crisis within the State of Washington and the nation by issuing his Executive Proclamations outside of his authority leading to severe deprivation of Plaintiffs' Constitutional and federal statutory rights to choose an option without penalty.

[70] 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) requires that individuals to whom the product is being administered are informed "of the option to accept or refuse administration of the product."

225. Congress expressly required Governor Inslee and all licensed healthcare facilities and workers in the State "to inform" Plaintiffs of their **legal rights** under the EUA statute, which are that they can either "accept" or "refuse" without penalty. Governor Inslee unlawfully amended the EUA statute by removing Plaintiffs 'legal option to "refuse" participation in a 100% federally funded program (CDC COVID-19 Vaccination Program) without consequence.

226. Governor Inslee exclusively relied on emergency medical countermeasures for compliance, which are also under PREP Act authority. The PREP Act also expressly restricts Governor Inslee from issuing laws, regulations, and ordinances that conflict with or interfere with the statute's provisions.[71]

227. Congress expressly wrote into legislation that Governor Inslee could not establish a "legal requirement" "of the covered countermeasure, **or to any matter** included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 et seq.]" (emphasis added). The "any matter" directly links to the authority of Plaintiffs to either accept or refuse the medical countermeasures without consequence under 21 U.S.C. §360bbb-3 (Federal Food, Drug, and Cosmetic Act).

228. Governor Inslee intentionally ignored the PREP Act's restriction of his authority, even though the CDC Provider Agreement provides, "Coverage under the Public Readiness and

[71]" Preemption of State law During the effective period of a declaration under subsection (b)…no State or political subdivision of a State may establish, enforce, or continue in effect with respect to a covered countermeasure any provision of law or legal requirement that— (A) is different from, or is in conflict with, any requirement applicable under this section; and (B) relates to the…administration…of the covered countermeasure, or to any matter included in a requirement applicable to the covered countermeasure under this section or any other provision of this chapter, or under the Federal Food, Drug, and Cosmetic Act [21 U.S.C. 301 et seq.]." - 42 USC 247d-6d(b)(8) See https://www.fda.gov/regulatory-information/search-fda-guidance-documents/emergency-use-authorization-medical-products-and-related-authorities#preemption

Emergency Preparedness (PREP) Act extends to Organization **if** it complies with the PREP Act and the PREP Act Declaration of the Secretary of Health and Human Services"[72](Emphasis added). Clearly, Governor Inslee did not comply with the PREP Act; therefore, immunity protections under the PREP Act do not extend to the State.

229. Although the PREP Act does not provide for a private right of action, the PREP Act's restrictions demonstrate that Governor Inslee acted under fraudulent pretense. Thus, his Proclamations were illegal and directly led to Plaintiffs 'legal, financial, and health injuries. Therefore, those injuries provide for a private right of action under 42 U.S.C. §1983 and other federal and state laws (see discussion, *infra*). Governor Inslee deceptively used the fact that the COVID-19 EUA drugs were granted expanded access protocols under 21 U.S.C. §360bbb-3 and the PREP Act as the means to push his political agenda while ignoring the restrictions contained within those same statutes.

230. It is well-established Washington State common law that Plaintiffs exercising a legal right may not be terminated or prohibited from employment for exercising such a right.[73]

231. It cannot be disputed that Plaintiffs considering participation in a medical countermeasure authorized under the EUA statute have the exclusive legal right to determine which option to choose, and Congress prohibits all other authorities from interfering with that choice.

232. Chaos has reigned within Washington State because Governor Inslee completely disregarded the right of Plaintiffs to be treated equally before the law as guaranteed to them under the Fourteenth Amendment. The "option" is the right. Governor Inslee knew that Plaintiffs were

[72] CDC COVID-19 Vaccination Program Provider Agreement

[73] *Becker v. Community Health Systems*, 184 Wn.2d 252, 359 P.3d 746 (2015); *Rose v. Anderson Hay & Grain Co.*, 184 Wn.2d 268, 358 P.3d 1159 (2015), and *Rickman v. Premera Blue Cross*, 184 Wn.2d 300, 358 P.3d 1153 (2015)

afforded the option, yet by fiat rule, he chose to demote Plaintiffs to second-class citizens for no other reason than he disagreed with their choice.

233. Governor Inslee declared by his actions that he, not the United States Congress, would determine which option Plaintiffs would choose. Such wanton disregard for the U.S. Constitution and its Separation of Powers Doctrine is unheard of in the modern-day Republic. It results in an actionable offense against Plaintiffs for which they are entitled to seek redress.

234. Governor Inslee's wanton disregard for the rights of Plaintiffs led him to conceal their right to refuse without consequence to such a degree that securing Plaintiffs 'substantive and procedural Due Process rights was legally impossible. **If persons in authority, such as Governor Inslee, refuse to acknowledge rights conferred upon Plaintiffs by valid acts of Congress, then due process is legally impossible to secure.**

235. Governor Inslee's Proclamation violates the well-established Unconstitutional Conditions Doctrine. The Supreme Court has held that a person "may not barter away his life or his freedom, or his substantial rights" (*Home Ins. Co. of New York v. Morse*, 87 U.S. 455, 451 (1874))

236. The State of Washington holds licensing power over the right of Plaintiffs to enjoy their profession, and Governor Inslee unlawfully utilized the powers of his office to prevent Plaintiffs from enjoying the equal protection of laws because of his personal preference in direct violation of his oath of office.

237. Governor Inslee's Proclamations require Plaintiffs to surrender their Constitutional Rights of Equal Protection of Laws and Due Process as a condition for continuing employment in their chosen profession. The US Supreme Court held:

> "It would be a palpable incongruity to strike down an act of state legislation which, by words of express divestment, seeks to strip the citizen of rights guaranteed by the federal Constitution, but to uphold an act by which the same result is accomplished under the guise of a surrender of a right in exchange for a valuable privilege which the state threatens otherwise to withhold. It is not necessary to challenge the proposition that, as a general rule, the state, having power to deny a privilege altogether, may grant it upon such conditions as it sees fit to impose. But the power of the state in that respect is not unlimited; and **one of the limitations is that it may not impose conditions which require the relinquishment of constitutional rights.** If the state may compel the surrender of one constitutional right as a condition of its favor, it may, in like manner, compel a surrender of all. **It is inconceivable that guaranties embedded in the Constitution of the United States may thus be manipulated out of existence** (emphasis added)."[74]

238. Governor Inslee's Proclamations established a condition that Plaintiffs must inject an investigational new drug into their bodies before October 18, 2021 to continue their employment under the State's licensing requirements. The U.S. Supreme Court held:

> "Broadly stated, the rule is that the right to continue the exercise of a privilege granted by the state cannot be made to depend upon the grantee's submission to a condition prescribed by the state which is hostile to the provisions of the federal Constitution.[75]

239. Governor Inslee unlawfully utilized the powers of his office to "produce a result which the State could not command directly" (*Speiser v. Randall*, 357 U.S. 513 (1958)). The State cannot demand by law that Plaintiffs inject an investigational new drug into their bodies[76] as a condition to participate in employment, education, the National Guard, or any licensed profession the State oversees, including healthcare.

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[74] *Frost Trucking Co. v. R.R. Com*, 271 U.S. 583, 593-94 (1926)

[75] *Western Union Tel. Co. v. Kansas*, 216 U.S. 1, 47, 48 S., 30 S. Ct. 190; *Western Union Tel. Co. v. Foster*, 247 U.S. 105, 114, 38 S. Ct. 438, 1 A. L. R. 1278. (*U.S. v. Chicago, M., St. P. &P. Railway Co*., 282 U.S. 311, 328-329 (1931)).

[76] 45 CFR § 46.116, 45 CFR §46.122, the Belmont Report (45 CFR § 46.101(c,i)), 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, PREP Act, 10 U.S.C. §980.

240. Because the HHS Secretary prescribed research conditions meeting 45 CFR 46 requirements in each COVID-19 EUAs, Governor Inslee was bound to obtain Plaintiffs 'legally effective informed consent. This consent requirement applies to any person acting on behalf of the sponsor[77] of the emergency medical countermeasure (e.g., COVID-19 drugs, masks, testing articles) program and not only to persons administering the product.

241. 45 CFR § 46.116 and the Belmont Report describe the conditions by which legally effective informed consent must be obtained. The burden on Governor Inslee when presenting Plaintiffs with the offer to participate in the federal government's property requires him to ensure Plaintiffs are not under "sanctions, "coercion," "undue influence," or "unjustifiable pressures" to participate.[78]

242. Governor Inslee deprived Plaintiffs 'of legally effective informed consent rights by establishing a "sanction" for non-participation in investigational new drugs under "coercive" conditions. Moreover, Governor Inslee knew of the requirement to obtain Plaintiffs 'legally effective informed consent by volunteering to participate in the CDC COVID-19 Vaccination Program Provider immunization project.[79]

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[77] The Federal Government is the sole sponsor of all COVID-19 EUA drugs, biologics, and devices, paying for 100% of their costs. Therefore, Washington State is acting on behalf of the sponsor when establishing mandatory participation in the government's property.

[78] The publication of the Belmont Report was a required condition of the 1974 National Research Act. Congress required the HHS Secretary to act on the Belmont Report and issue regulations providing for the Protection of Human Subjects. 45 CFR §46.116 is the only federal definition of informed consent, and therefore, whether federal law explicitly or implicitly requires informed consent, this is the only known meaning of that requirement demonstrating the intent of Congress.

[79] CDC COVID-19 Vaccination Program Provider Agreement (Exhibit A) Clause 12(a) states, "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine." Therefore, 21 U.S.C. §360bbb-3 (under Food, Drug, and Cosmetic Act) requires adherence to 45 CFR §46.116 and the Belmont Report.

243. Governor Inslee, acting with moral turpitude, placed Plaintiffs under moral duress to inject an unlicensed investigational drug into their bodies, knowing they relied on their chosen profession to access living wages and would be under extreme emotional distress to comply with his unlawful usurpation of authority.[80]

244. Long before the COVID-19 pandemic started, Washington State provided HHS with an "assurance" that it would comply with 45 CFR 46 and the Belmont Report anytime it offered an individual participation in an investigational new drug.

245. This assurance is a requirement by the Federal Government of Washington State as a condition for the State to receive federal funds to be expended on research activities.[81]

246. The agreement is why the CDC chose to use the State's existing immunization agreement because Governor Inslee was already bound to protect the rights of individuals under his authority when it agreed to the Federal Wide Assurance[82] (FWA) agreement administered under the Office of Human Rights Protection within HHS.

247. In exchange for the assurance to comply with the ethical principles of the Belmont Report and 45 CFR 46 when involving humans with INDs, HHS assigned Washington State FWA00000327, denoting a legally binding agreement between the State of Washington and the United States Government for the explicit benefit of third-party participants.

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[80] Moral duress consists of imposition, oppression, undue influence, or the taking of undue advantage of the business or financial stress or extreme necessity or weakness of another. *Lafayette Dramatic Productions v. Ferentz*, 306 Mich. 193, 9 N.W.2d 57, 66; See also Black's Law Dictionary, Sixth Edition, p. 1008.

[81] 45 CFR § 46.122

[82]" The Federal Wide Assurance (FWA) is the only type of assurance currently accepted and approved by OHRP. Through the FWA, an institution commits to HHS that it will comply with the requirements in the HHS Protection of Human Subjects regulations at 45 CFR part 46." - 1.Office. Federalwide Assurances (FWAs). HHS.gov. Published June 16, 2009. Accessed August 31, 2023. https://www.hhs.gov/ohrp/federalwide-assurances-fwas.html#:~:text=The%20Federalwide%20Assurance%20(FWA)%20is,at%2045%20CFR%20part%2046.

248. Governor Inslee deprived Plaintiffs 'third-party beneficiary rights under the FWA agreement when he issued the requirement that Plaintiffs inject investigational new drugs into their bodies by October 18, 2021 as a condition to continue employment in their chosen healthcare profession.[83]

249. In direct violation of 45 CFR §46.116 and the Belmont Report and deprivation of Plaintiffs 'rights thereunder, Governor Inslee purposefully placed Plaintiffs under "sanctions," "coercion," and "undue influence" to participate in an EUA medical countermeasure.

250. Governor Inslee pledged that Washington State would protect individuals considering participation in the CDC COVID-19 Vaccination Program. Before the ink dried from his affirmation, he turned around and deprived the third-party beneficiary rights belonging to Plaintiffs under that contract and the State's FWA.

251. Although the Biden Administration purchased all COVID-19 EUA drugs, it is not exempt from 21 U.S.C. §360bbb-3 requirements. Moreover, simply because the CDC has a program whereby Washington State can access the Federal Government's EUA property does not exempt the State from complying with its legal obligations under 21 U.S.C. §360bbb-3. The CDC COVID-19 Contract is an additional layer of responsibility for organizations to comply with, not an exemption from their legal obligations under 21 U.S.C. §360bbb-3.

252. Therefore, Governor Jay Inslee knew that 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) contained a required condition of him "to ensure that individuals to whom the product is administered are informed— of the option to accept or refuse administration of the product."

[83] 45 CFR §46.116

253. At all times pertinent, Governor Jay Inslee concealed this right from Plaintiffs in his Proclamations and intentionally refrained from guiding the healthcare community on this right because it undermined his desire to push an experimental biomedical research project onto Plaintiffs outside of their free will and voluntary consent.

254. Governor Inslee's statement in Proclamations 21-14 and 21-14.1 that: "COVID-19 vaccines are safe and effective" was meant to deceptively mislead Plaintiffs into participating in drugs having no FDA legal indication that the drugs are either safe or effective.[84] Moreover, such a statement directly violates federal law.[85]

255. Governor Inslee issued a misleading statement meant to compel participation under false pretense when stating, "COVID-19 vaccines were evaluated in clinical trials involving tens of thousands of participants and met the U.S. Food & Drug Administration's (FDA) rigorous scientific standards for safety, effectiveness, and manufacturing quality needed to support emergency use authorization; and, to date, more than 346 million doses of COVID-19 vaccines have been given in the United States with 8.2 million of those doses administered in Washington, and serious safety problems and long-term side effects are rare."

256. The Governor's statement lacked information Plaintiffs would want to know when considering participation.[86] For example, COVID-19 mRNA drugs also had historical reports of adverse events, were not manufactured according to standards licensed drugs are manufactured, and had heart-related and blood clotting issues that were not common side effects of a typical

[84]" Unless and until FDA properly classifies AVA [an anthrax vaccine] as a safe and effective drug for its intended use, an injunction shall remain in effect prohibiting defendants 'use of AVA on the basis that the vaccine is either a drug unapproved for its intended use or an investigational new drug…" *Doe v. Rumsfeld*, 341 F. Supp. 2d 1 (D.D.C. 2004)
[85] "A sponsor or investigator, or any person acting on behalf of a sponsor or investigator, shall not represent in a promotional context that an investigational new drug is safe or effective for the purposes for which it is under investigation or otherwise promote the drug." 21 CFR 312.7(a)
[86] 45 CFR 46.116(a)(4)

"vaccine." Additionally, he failed to inform recipients that Pfizer's BioNTech COVID-19 Vaccine trial lost 93% of its participants by the sixth month of a 24-month trial. Most importantly, Governor Inslee intentionally concealed that a person choosing to volunteer as an EUA drug recipient is prohibited from seeking any meaningful judicial relief when injured by the medical countermeasure, specifically because they are under PREP Act authority.

257. Most concerning is Governor Inslee's concealment of the legally binding agreement Plaintiffs must agree to before injecting a 21 U.S.C. §360bbb-3 IND into their bodies. Congress was explicit that any person receiving a medical countermeasure under 21 U.S.C. §360bbb-3 protocols under PREP Act authority must agree to the following terms and conditions, including, but not limited to:

A. forfeiture of civil litigation rights resulting from injuries,[87]

B. allow their private identifiable information to be collected and used for a variety of purposes by unknown persons,[88]

C. allow their involvement with the EUA product to be cataloged by various persons for unknown purposes,

D. allow the data collected about their adverse events to be utilized by researchers for unknown purposes and eternity,[89]

E. assume greater risks to their safety, health, and legal rights.[90],[91]

[87] PREP Act forfeits all civil actions for damages in most situations.

[88] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Program requires manufacturers and/or emergency stakeholders to obtain private identifiable information.

[89] Each EUA and/or the CDC COVID-19 Vaccination Program Provider Program requires manufacturers and/or emergency stakeholders to monitor, report and study a variety of adverse reactions to EUA products.

[90] 21 U.S.C. §360bbb-3 requires potential recipients to be made aware of the risks, alternatives, and the fact that the product is only authorized by the Secretary under emergency conditions. These elements provide potential recipients with the required information to make a quality and legally effective decision to consent. Therefore, consent means the individual agrees to assume more than minimal risk.

[91] 21 CFR 50.3(k) (Protection of Human Subjects; Definitions) defines minimal risk as "the probability and magnitude of harm or discomfort anticipated in the research are not greater in and of themselves than those ordinarily encountered in daily life or during the performance of routine physical or psychological examinations or tests."

258. Governor Inslee violated RCW 7.7.060 by unlawfully mandating medical treatment for Plaintiffs, vitiating the statute's informed consent protocols.

259. Governor Inslee deprived the Plaintiffs' rights under RCW 69.77.050, requiring specific informed consent protocols for administering investigational new drugs.

260. Governor Inslee's requirement that Plaintiffs must provide proof of being injected with an unlicensed investigational drug was a severe deprivation of the privacy rights of Plaintiffs.

261. Plaintiffs choosing to be injected with unlicensed medical products is a private matter, and not even the Governor is empowered to force "public disclosure of Plaintiff's private facts."[92] All citizens have the right to accept or refuse investigational (i.e., experimental) products as a private affair, and intrusion into that private matter is "highly offensive"[93] to Plaintiffs.

262. Governor Inslee stated, "Workers for State Agencies, Workers for operators of Educational Settings, and Health Care Providers are not required to get vaccinated against COVID-19 under this Order if they are unable to do so because of a disability or if the requirement to do so conflicts with their sincerely held religious beliefs, practice, or observance."

263. The requirement to obtain an exemption from medical experimentation is an unlawful misrepresentation of authority by the Governor.

264. Congress was explicit in that only the HHS Secretary has the authority to establish the conditions under which 21 U.S.C. §360bbb-3 products are administered. Not even the Secretary can require any person to participate in any activity under the section of law. Therefore, the mandate to seek exemption violated Plaintiffs' absolute authority to autonomously determine participation in EUA or PREP Act products outside of Governor Inslee's interference.

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[92] *Reid v. Pierce County*, 136 Wn. 2d 195, 136 Wash. 2d 195, 961 P.2d 333 (Wash. 1998) and see *Hearst Corp. v. Hoppe*, 90 Wn. 2d 123, 90 Wash. 2d 123, 580 P.2d 246 (Wash. 1978)

[93] *Adams v. King Cty.*, 164 Wash. 2d 640, 662, 192 P.3d 891, 902 (2008)

265. Governor Inslee ignored his constitutional restrictions, legal obligations, and his oath of office to protect the rights, privileges, and immunities of Plaintiffs. With utter disregard for the rule of law, Governor Inslee assumed a power not granted to him for the intentional purpose of causing the Plaintiffs to surrender their constitutional rights resulting from their fears of economic loss.

266. However, the following statement demonstrates Governor Inslee's moral turpitude:

> "Workers for State Agencies, Workers for operators of Educational Settings, and Health Care Providers are prohibited from claiming an exemption or accommodation on false, misleading, or dishonest grounds, including by providing false, misleading, or dishonest information to a State Agency, operator of an Educational Setting, or operator of a Health Care Setting when seeking an accommodation."

267. Governor Inslee's entire 21-14.1 Proclamation is built upon "false," "misleading," and "dishonest grounds" "by providing false, misleading, or dishonest information" to the general public. The moral turpitude of his statement demonstrates the utter disregard for Plaintiffs' rights, the Constitution, and his oath of office.

268. Governor Inslee harassed, intimidated, publicly humiliated, and used the powers of his office in an attempt to cause Plaintiffs to surrender the Constitutional Protections, Statutory rights, and third-party contract benefits. What he could not directly achieve by law through his State's legislature, he indirectly achieved by fiat by ordering healthcare facilities to financially destroy the careers, families, dreams, goals, and lives of Plaintiffs for no other reason than he disagreed with their 21 U.S.C. §360bbb-3 chosen option of refusing an EUA investigational drug.

269. Governor Inslee cannot produce a treaty, statute, regulation, or other legal power affording him authority to ignore Congress, abuse the Constitution, or defy his oath of office when requiring anyone to inject an investigational new drug into their body as a condition of anything.

270. Congress preempted Governor Inslee's actions, restricted his authority, denied his power, and still, Governor Inslee acted as if the rule of law did not apply to him, his office, or those state actors under his authority.

271. Governor Inslee's actions are more akin to an authoritarian dictatorship than a duly elected officer acting for the public's benefit by upholding his office's oath to respect Plaintiffs ' rights as guaranteed to them by the United States Constitution and the Washington State Constitution.

272. Governor Inslee's actions directly led to Plaintiffs experiencing life-altering emotional trauma, financial destruction, and loss of life's goals, dreams, and aspirations. Governor Inslee intentionally inflicted emotional distress on the lives of Plaintiffs for the express purpose of deceptively compelling them to surrender their Constitutional protections and statutory rights.

Shriners Hospitals for Children

273. Shriners Hospitals for Children is a network of not-for-profit medical facilities incorporated in Colorado.

274. At all times pertinent, Plaintiffs were under the authority of Shriners PolicyMakers and were employed at Shriners Hospitals for Children – Spokane located at 911 W 5th Avenue, Spokane, Washington.

275. Shriners PolicyMakers signed the CDC COVID-19 Vaccination Agreement, assured HHS it would comply with its FWA00025698 agreement, established institutional review boards under 45 CFR Part 46 authority, and instituted and enforced Shriners Hospitals for Children's COVID-19 policy.

276. Although Shriners Hospitals for Children, Shriners Hospitals for Children — Spokane, and its PolicyMakers, at other times and in other circumstances, are private parties, they

acted under color of law when, as collaborators with the State of Washington pursuant to the Provider Agreement, penalized Plaintiffs for refusing to inject one of the mandated unlicensed investigational drugs into their bodies. The Supreme Court and Ninth Circuit Court of Appeals utilize several tests to ascertain when a private party is engaged in state action. Shriners Hospitals for Children and/or Shriners Hospital for Children – Spokane are state actors under the (1) Public Function Test,[94] (2) State Compulsion Test,[95] (3) Symbiotic Relationship Test,[96] and (4) Customs Test.[97]

277. On September 14, 2021, William S. Bailey, Imperial Potentate of Shriners International, Jerry G. Gantt, Chairman of the Board of Trustees of Shriners Hospitals for Children, and John P. McCabe, Executive Vice President of Shriners Hospitals for Children, issued and signed an unlawful directive to "all Shriners Hospitals for Children Employees and Contract Staff" regarding a new company policy titled, "COVID-19 Vaccine Policy."

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[94] The CDC COVID-19 Vaccination Program was exclusively a government function for the public, in which no private party could participate, administrate, or even charge the public for the Program. Though the Program was novel and new, it fits the historical definition of a governmental public function.

[95] The State issued a mandate impacting Plaintiffs' employment. Shriners Hospital – Spokane is a state actor under this test pursuant to *Blum v. Yaretsky*, 457 U.S. 991, 1004 (1982) (citing *Flagg Bros., Inc. v. Brooks*, 436 U.S. 149, 166 (1978); *Jackson*, 419 U.S. at 357; *Adickes v. S.H. Kress & Co*., 398 U.S. 144, 170 (1970); *Moose Lodge No. 107 v. Irvis*, 407 U.S. 163, 173 (1965)).

[96] Shriners Childrens and Washington State were required by the federal government to conduct medical research and obtain Plaintiffs' legally effective informed consent. These requirements demonstrate a symbiotic relationship between the State and Shriners Childrens pursuant to: *Brunette v. Humane Society of Ventura County*, 294 F.3d 1205 (9th Cir. 2002): "Burton (*Burton v. Wilmington Pkg. Auth*, 365 U.S. 715, 81 S. Ct. 856 (1961)) teaches that substantial coordination and integration between the private entity and the government are the essence of a symbiotic relationship. Often significant financial integration indicates a symbiotic relationship. See *Rendell-Baker v Kohn*, 457 U.S. at 842-43, 102 S.Ct. 2764; *Vincent v. Trend W. Tech. Corp*., 828 F.2d 563, 569 (9th Cir. 1987). For example, if a private entity, like the restaurant in Burton, confers significant financial benefits indispensable to the government's "financial success," then a symbiotic relationship may exist. *Vincent*, 828 F.2d at 569. A symbiotic relationship may also arise by virtue of the government's exercise of plenary control over the private party's actions. See *Dobyns v. E-Systems, Inc*., 667 F.2d 1219, 1226-27 (5th Cir. 1982) (finding symbiotic relationship where the government-controlled a private peacekeeping force engaged in a government-directed field mission in the Sinai Peninsula).

[97] The Supreme Court noted in *Adickes v. S. H. Kress & Co*., 398 U.S. 144 (1970) that the "Petitioner will have established a claim under §1983 for violation of her equal protection rights if she proves that she was refused service by respondent because of a state-enforced custom…" (emphasis added). Governor Inslee, Attorney General Bob Ferguson, and Shriners PolicyMakers developed a state custom whereby a person's 21 U.S.C. §360bbb-3 statutory rights can be ignored "as if" they do not exist.

278. The memo stated in part:

A. "Last week the Joint Boards approved a policy requiring nearly everyone in our organization to get fully vaccinated against COVID-19."

B. "With the recent FDA approval of the Pfizer product, we know that a safe and effective vaccine exists. FDA approval of the Moderna and Johnson & Johnson vaccines is expected soon. Science has shown that overwhelmingly, these vaccines protect people from hospitalization, ICU-level care and death. For everyone's safety, our Joint Boards did not take a vaccine requirement policy under consideration until FDA approval was granted. They then initiated a thorough and deliberative process to determine the appropriate course of action for our organization."

C. "Everyone at every Shriners Hospitals for Children location in the United States and Canada, including Headquarters, must comply with the policy. All employees, contract employees, vendors, and volunteers who have a need to enter a building owned and/or operated by Shriners Hospitals for Children are required to be fully vaccinated against the COVID-19 virus."

D. "Everyone will need their first shot of the vaccination series by October 11, and must be fully vaccinated or have an appropriate exemption approved by December 6."

E. "It is a requirement of employment with Shriners Hospitals for Children that all eligible persons be fully vaccinated. Those few who receive exemptions will be required to follow strict protocols that are anticipated to include masking with a special N95 mask and weekly COVID testing."

F. "The requirements for medical or religious exemptions are specific. Talk to your local Human Resources team about your personal situation. To apply for an exemption, you must submit supporting documentation as well as a completed request form. All exemption requests will be vetted by a team including corporate leadership."

279. Therefore, after the Shriners PolicyMakers "initiated a thorough and deliberative process to determine the appropriate course of action" regarding the new COVID-19 policy, Shriners PolicyMakers issued a policy requiring Plaintiffs to inject a COVID-19 drug into their body before October 11 or seek an exemption from the policy as a condition of continued employment with its organization.

280. Moreover, although the FDA licensed COMIRNATY for general commercial marketing, it was not available to Plaintiffs, and therefore, Shriners PolicyMakers intentionally required all employees to inject an unlicensed investigational new drug[98] into their bodies for policy compliance, drugs that had trillions of potential adverse reactions that the manufacturers, CDC, FDA, or Shriners had not researched, which could have had life-threatening complications to the health of Plaintiffs had they chosen to accept the injection.

281. Shriners PolicyMakers are legally sophisticated in laws applicable to the investigational new drug classification and know they are legally restricted from applying outside pressure on Plaintiffs to participate in federally funded research projects, much less sanctioning them for refusing to participate.

282. Shriners PolicyMakers signed and/or approved the CDC COVID-19 Vaccination Provider Agreement and knew of its legal requirements prohibiting them from mandating participation in the Program's activities.[99] Moreover, they agreed to conduct joint medical research activities with the State and obtain Plaintiffs' legally effective informed consent.

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[98] 21 CFR 312.3

[99] 12(a) of the contract states, "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine." 21 U.S.C. §360bbb-3 contains the option to accept or refuse, which Joint Board Members voluntarily agreed to adhere to when involving Plaintiffs in the CDC COVID-19 Vaccination Program.

283. Shriners PolicyMakers assured HHS that Shriners Hospitals for Children would **never** place an individual under "coercion," "undue influence," "sanction," or "unjustifiable pressure" to inject unlicensed drugs into their bodies, which were funded by the federal government when HHS awarded Shriners its Federal Wide Assurance Agreement Number (FWA00025698). Shriners PolicyMakers had to promise to comply with 45 CFR § 46.122 and the Belmont Report having intended benefits for Plaintiffs.

284. Shriners PolicyMakers have an institutional review board (IRB) under 45 CFR Part 46 and the Belmont Report protocols overseeing all investigational new drug administration protocols, which prohibited the application of their COVID-19 policy.

285. Shriners PolicyMakers knew of each COVID-19 EUA Scope of Authorization prohibiting mandatory participation. Even if the CDC did not implement the COVID-19 Vaccination Program, Shriners PolicyMakers knew that compliance with the EUA statute is a legal requirement they cannot violate.

286. Shriners PolicyMakers oversee a multi-million dollar research program utilizing the same laws governing the COVID-19 EUA drugs.[100]

287. Shriners PolicyMakers oversaw the administration of drugs and biologics administered to patients under 21 U.S.C. §360bbb *et seq.,* for decades[101] and always obtained a patient's informed consent because they knew the statute's requirements.

288. Despite knowing the laws, contracts, and agreements, Shriners PolicyMakers willfully and with wanton disregard for Plaintiffs' Constitutional and statutory rights issued an unlawful policy[102] on September 14, 2021, stating in part:

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[100] https://www.shrinerschildrens.org/en/research-and-expertise/research-at-shriners-childrens

[101] Hospitals routinely utilize INDs for cancer patients which require 21 U.S.C. §360bbb *et seq.,* authority.

[102] Exhibit E, Shriners COVID-19 Vaccine Policy

A. "All Shriners Hospitals for Children employees and all employees, contract employees, vendors, and volunteers who enter a building owned and/or operated by Shriners Hospitals for Children are required to be fully vaccinated against the COVID-19 virus."[103]

B. "Exemptions may be granted for a medical reason or a religious reason, which meet applicable standards, and is subject to reasonable accommodation."[104]

C. "If an exemption is granted, the exempted employee will be required to wear, as applicable, a NIOSH approved N95 respirator/mask or available equivalent mask such as a PAPR or elastomeric respirator/mask at all times during their scheduled shift. HCP will be fit-tested for the appropriate N95 mask or equivalent. The employee may remove the respirator/mask only when actively eating or drinking beverage/meals outside of direct patient care areas, and maintaining required social/physical distancing. If a religious or medical exemption is granted, the exempted healthcare personnel will be required to be tested for COVID-19 on a weekly basis or as frequently required under applicable CDC guidelines or state/local public health standards."[105]

D. "HCP with specific medical contraindications as defined by the COVID-19 vaccine manufacturer will be exempt from vaccination but require N95 masking and weekly COVID-19 vaccine testing."[106]

E. "If vaccination, proof of vaccination from outside provider or exemption has not occurred by vaccination deadline, EOH or HCP supervisor will send a written notification to informing them they

[103] In other words, all persons under their authority were required to inject unlicensed investigational drugs into their bodies as a condition to enter any Shriner's building despite Congress expressly restricting them from exercising such fiat authority.

[104] Congress provides Plaintiffs with the absolute right to accept or refuse said products without the unlawful condition of seeking out an exemption. This was a fraudulent misrepresentation of facts to Plaintiffs for the express purpose of deceiving them into surrendering their Constitutional and statutory rights.

[105] This requirement is a clear deprivation of Plaintiffs' equal protection and due process rights as guaranteed to them under the Fourteenth Amendment.

[106] There were no available vaccines in general commercial marketing. Investigational new drugs may not be promoted as being safe or effective. (21 CFR 312.7(a)) Therefore, no Plaintiff could be medically exempt from the mandate resulting from an act of pure fraud by Shriners PolicyMakers, placing Plaintiffs at severe health risks (*see, infra*).

are not in compliance with the SHC COVID-19 Vaccination for Healthcare Personnel policy, and are not on the SHC premises."[107]

F. "HCP shall have completed their first vaccination shot by October 11, 2021." "HCP shall have completed the vaccination series and be fully vaccinated by December 6, 2021."[108]

289. The policy, as applied, deprived Plaintiffs of their Fourteenth Amendment and statutory rights as follows:

A. Only persons choosing the option to refuse were penalized and not persons choosing the option to accept. Punishments included the requirement to wear masks under EUA and the PREP Act, as well as engaging in unwanted experimental medical testing[109] procedures and, ultimately, termination of their careers. These actions represent a clear violation of Plaintiffs' Fourteenth Amendment equal protection rights.

B. Shriners PolicyMakers only denied the substantive and procedural due process rights of persons refusing to participate in an EUA product or PREP Act countermeasure. At all times pertinent, Shriners PolicyMakers refused to acknowledge the well-established rights of Plaintiffs to refuse such products without consequence, making due process impossible to secure.

C. Shriners PolicyMakers intentionally deprived Plaintiffs of their statutory rights under 21 U.S.C. §360bbb-3 (the EUA statute).

[107] Willfully ignoring Plaintiffs' statutory rights, Shriners PolicyMakers demonstrated their moral turpitude by intentionally placing Plaintiffs under coercion and moral duress to compel participation in their unlawful policy outside of Plaintiffs' free will and voluntary consent.

[108] No COVID-19 drugs existed in 2021 or 2022 that the FDA licensed and also introduced into commerce for general commercial marketing. Therefore, it is indisputable that at all times pertinent Shriners PolicyMakers unlawfully relied on INDs for policy compliance.

[109] All COVID-19 tests were under the EUA statute and the PREP Act. All EUA products are investigational according to their intended purpose under an EUA. If the products are licensed for the intended use, then the HHS Secretary is restricted from issuing an EUA altogether.

D. Shriners PolicyMakers intentionally refused to obtain Plaintiffs' legally effective informed consent[110] in violation of 45 CFR § 46.116 and the Belmont Report.

E. Shriners PolicyMakers intentionally subjected Plaintiffs to medical experimentation in violation of Article VII of the ICCPR Treaty.

F. Shriners PolicyMakers intentionally failed to fulfill their duties under their FWA, IRB, and CDC COVID-19 Vaccination Program Provider Agreement, duties required by the federal government for the intended benefit of Plaintiffs.

290. Although the policy spans seven pages, Shriners PolicyMakers fraudulently concealed the legal rights of Plaintiffs to accept or refuse any product under the EUA statute or the PREP Act. As a matter of law and federal contractual obligations having third-party benefits for Plaintiffs, Shriners PolicyMakers were bound to inform Plaintiffs of their rights when presenting the COVID-19 Vaccination Program to them. Yet, not one time does Shriners PolicyMakers mention the EUA statute in the policy despite that statute being the only authority authorizing the use of the mandated investigational drugs.

291. Shriners PolicyMakers unlawfully concealed from all of its employees the legal terms and conditions they must agree to as established by Congress (see, *supra*) when choosing whether to participate in an EUA or PREP Act product, terms and conditions that have potentially severe legal consequences The purpose of the concealment was to coerce Plaintiffs into involuntarily surrendering their Constitutional and statutory rights under deceptive means.

292. Shriners PolicyMakers COVID-19 policy usurped the authority of the HHS Secretary by unlawfully amending the conditions established by the Secretary in the Scope of

[110] Legally effective informed consent requires an environment free of outside pressure before the offer to participate in the product is presented to an individual. - The Belmont Report and 45 CFR § 46.116

Authorization for each COVID-19 EUA drug. Moreover, Shriners PolicyMakers ignored the authority of the U.S. Congress by requiring the very condition (mandatory) that Congress prohibits. The Supremacy Clause prospectively preempted Shriners PolicyMakers' authority to establish their COVID-19 policy as applied. The policy was patently illegal, and Shriners PolicyMakers knew it.

293. On August 11, 2021, Kristin Monasmith, Director of Marketing, sent out a company-wide email[111] to all Spokane, WA employees stating in part:

A. "Gov. Jay Inslee announced a requirement for most state workers, and on-site contractors and volunteers to be vaccinated against COVID-19 as a condition of employment."

B. "State employees and workers in private health care and long-term care settings will have until October 18 to be fully vaccinated, as per the Proclamation."

C. "Coming from the Governor and the state of Washington, the Spokane Hospital is required to abide by the state mandate and follow applicable laws. Shriners Hospitals for Children does not have the ability to adapt or vary from the statements within the proclamation."

294. The statement that "Shriners Hospitals for Children does not have the ability to adapt or vary from the statements within the proclamation" was patently untrue for the following reasons:

A. Governor Inslee's compliance date required Shriners PolicyMakers to rely exclusively on EUA investigational drugs for compliance. Shriners Policymakers were required to comply with federal law that preempted Governor Inslee's authority.

[111] See Exhibit F, Shriners August 11, 2021 email

B. The express preemption language of the PREP Act[112] prohibited Shriners PolicyMakers from establishing conditions conflicting or interfering with the provisions of the statute.

C. Shriners PolicyMakers signed or authorized the CDC COVID-19 Vaccination Program Provider Agreement prohibiting them from utilizing EUA drugs to comply with Governor Inslee's mandate.

D. Shriners Hospitals for Children had IRB and FWA obligations that preempted Governor Inslee's authority.

E. Governor Inslee's 21-14-1 Proclamation stated, "**ADDITIONALLY**, the specific prohibitions in this Proclamation are severable and do not apply to the extent that compliance with a prohibition would violate (1) any U.S. or Washington constitutional provision; (2) federal statutes or regulations; (3) any conditions that apply to the state's receipt of federal funding; (4) state statutes; or (5) applicable orders from any court of competent jurisdiction."

295. Shriners PolicyMakers were free and, in fact, instructed to protect Plaintiffs' rights by complying with federal law, their contractual obligations, and HHS's legal obligations unabated, but they intentionally failed to do so.

296. Although Governor Inslee violated his own Proclamation, Shriners PolicyMakers were not required or authorized by law to follow suit and only did so because the Governor established a state custom of lawlessness, affording Shriners PolicyMakers a belief that the State of Washington would not enforce the laws on the books.

297. Shriners PolicyMakers engaged in their lawless activity to participate in the rich flow of cash resulting from the CDC COVID-19 Vaccination Program.

[112] 42 USC 247d-6d(b)(8) The PREP Act preemption directly extends to any applicable requirement under 21 U.S.C. §360bbb-3 such as the option to accept or refuse.

298. Shriners PolicyMakers were legally bound, and in fact instructed, to not comply with Governor Inslee's Proclamation if it was unlawful as applied. Shriners PolicyMakers had full authority and legal power to ignore Governor Inslee's mandates since it required Shriners PolicyMakers to violate their legal obligations. However, Shriners PolicyMakers' personal desires outweighed their moral conviction to obey the law, conspiring with Governor Inslee's Proclamation as cover for their lawlessness.

299. Shriners' new mantra became: Ignore the Constitution, ignore federal laws, ignore a signed contract, deprive Plaintiffs of their rights, and collect the cash at all costs.

300. The moral turpitude of Shriners PolicyMakers is painfully realized when Plaintiffs Ginger Bennett, Jane Roberts, Michelle Andrews, and Jon Alleman sought out a medical exemption, and all were denied because Shriners PolicyMakers established a scheme relying on contraindications from EUA drugs that were not fully established.

301. Requiring Plaintiffs to request a medical or religious exemption from mandatory injections of investigational drugs under the EUA statute and the PREP Act is a violation of federal law and their signed CDC COVID-19 Vaccination Provider Agreement. Plaintiffs' right to accept or refuse such products is absolute, and no person in the United States of America has the authority to interfere with their preferred option, much less mandate they seek out an exemption.

302. Plaintiff Ginger Bennett's licensed medical doctor informed Shriners PolicyMakers in unambiguous language that the "patient experienced severe reaction to previous non-covid vaccine. It is impossible to know which ingredient she reacted to. She has been advised to avoid vaccines due to potentially life threatening reaction." Moreover, that past "severe reaction" was a vaccine administered by Shriners Hospitals for Children – Spokane, of which they were well aware.

303. Pfizer and the FDA informed Shriners PolicyMakers to "not administer Pfizer-BioNTech COVID-19 Vaccine to individuals with known history of a severe allergic reaction (e.g., anaphylaxis) to any component of the Pfizer-BioNTech COVID-19 Vaccine.[113]" Still, Shriners Policymakers informed Ms. Bennett that, "…in regards to your request from the COVID immunization requirement for medical reasons. Careful consideration from every angle has been given to this request and unfortunately, Shriners Hospitals for Children is unable to accommodate your request for exemption. We can assure you your request has been thoroughly reviewed by local human resources and appropriate corporate leadership." Shriners PolicyMakers allowed "local human resources" to engage in the unlawful practice of medicine and take action contradictory to Ms. Bennett's licensed doctor and her health.

304. Shriners PolicyMakers established a medical exemption form [114] specific to COVID-19 and required employees to initial several statements under duress as a condition to have their exemption from investigational drug use considered.

305. One of the statements Shriners PolicyMakers required Plaintiffs to sign seeking a medical exemption is, "I understand by declining this vaccination, if I am granted an accommodation I will be required to follow CDC, state and local Health Department and Shriners Hospital guidelines regarding face coverings, physical distancing and/or testing or other reasonable accommodations as determined."

306. The condition placed upon Plaintiffs deprived them of their Fourteenth Amendment rights of equal protection and due process. An individual refusing an EUA/PREP Act product cannot be treated differently than an individual accepting the product.

[113] See Exhibit G, Pfizer-BioNTech COVID-19 Vaccine drug fact sheet
[114] See Exhibit H, Ginger Bennett's medical exemption form

307. Shriners PolicyMakers compelled Plaintiffs, under intense emotional strain, to testify with their signature that they prospectively agreed to certain conditions if they were exempted from Shriners PolicyMakers' requirement to be injected with an EUA investigational drug. In other words, Shriners PolicyMakers held the proverbial gun to Plaintiffs' head and said if you want us to consider your medical condition, you must first sign this document waiving your Fourteenth Amendment rights. If you do not, we will deprive you of your Constitutional and federal statutory rights by terminating your employment irrespective of your medical condition or rights.

308. The policy and corresponding actions of Shriners PolicyMakers demonstrate a culture of moral turpitude rampant throughout Shriners Hospitals for Children, where the rule of law is ignored, the rights of Plaintiffs are trampled, and no one is held accountable for their lawlessness.

309. Shriners PolicyMakers, with willful and wanton disregard for the rights, safety, and welfare of the Plaintiffs, intentionally ignored their lawful obligations, contractual duties, and federal agreements for no other reason than the age-old sin of greed. Worse yet, they used a pandemic to hide their malfeasance and willfully applied as much pressure on Plaintiffs as one in their positions of power could to set them as an example to all others that no one under their authority would ever have the freedom to exercise their Constitutional and statutory rights without severe life-altering consequences.

310. Shriners PolicyMakers intentionally inflicted emotional distress upon Plaintiffs by destroying their dreams, careers, goals, housing, education, family life, healthcare, retirement, and feelings of dignity and equal treatment. Shriners PolicyMakers subjected Plaintiffs to medical investigation outside of their free will and voluntary consent.

311. Shriners PolicyMakers engaged in lawless activity that shocked the conscience, was outrageous, intolerable, and extreme, and placed Plaintiffs in severe emotional distress, fearing for their lives and livelihoods. Such debased leadership is unheard of in modern societies and exceeds the bounds of decency.

VII. Legal Claims

312. The facts described above constitute a deprivation of several rights guaranteed to Plaintiffs by the United States Constitution, federal statutes, and treaties. These deprivations are actionable under 42 U.S.C. § 1983 because the Defendants acted under color of state law when issuing their COVID-19 vaccination requirements and administrating the CDC COVID-19 Vaccination Program pursuant to the CDC COVID-19 Vaccination Program Provider Agreement and the federal statutes cited therein.

313. Court precedent demonstrates that federal statutes and regulations with rights conferring language are enforceable under 42 U.S.C. §1983.[115]

314. Defendants were, and are, restricted from attempting to use state law to amend the above-referenced statutes, regulations, treaties, agreements, and contracts due to the Supremacy Clause Doctrine. The Supremacy Clause Doctrine, and the express preemption language in the PREP Act and 21 U.S.C. §360bbb-3, restrict public and private employers from using state laws to require individuals to participate in any EUA or PREP Act activity or use any EUA or PREP Act product. This extends to any at-will employment law, doctrine, or custom an employer would otherwise claim as the right to interfere in the CDC Vaccination Program, 21 U.S.C. §360bbb-3, or PREP Act protocols and to amend conditions established by Congress for Plaintiffs' benefit.

[115] *Maine v. Thiboutot*, 448 U.S. 1 (1980), the court held that "Even were the language ambiguous, however, any doubt as to its meaning has been resolved by our several cases suggesting, explicitly or implicitly, that the §1983 remedy broadly encompasses violations of federal statutory as well as constitutional law." See also, *Health and Hospital Corporation of Marion Cty. V. Talevski.*

COUNT ONE

42 U.S.C. § 1983 – Subjected to Investigational Drug Use

315. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

316. The CDC COVID-19 Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR Part 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, 10 U.S.C. § 980, EUA Scope of Authorization letters, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

317. 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III) (the EUA statute) contains a required condition of the Secretary "to ensure that individuals to whom the product is administered are informed — 'of the option to accept or refuse administration of the product.'"

318. 21 U.S.C. §360bbb-3, the CDC COVID-19 Vaccination Program Provider Agreement, and each EUA's Scope of Authorization contains research conditions for COVID-19 medical products meeting 45 CFR 46.102(l)'s definition of research requiring adherence to 45 CFR § 46.101[116] *et seq.*

319. "Before involving a human subject in research covered by this policy, an investigator shall obtain the legally effective informed consent of the subject or the subject's legally authorized representative." 45 CFR 46.116(a)(1)

320. 45 CFR § 46.116 and the Belmont Report contain the only known definition of legally effective informed consent.

[116]" This policy applies to all research involving human subjects conducted, supported, or otherwise subject to regulation by any Federal department or agency" 45 CFR 46.101(a).

321. 45 CFR 46.116(b)(8) states: "A statement that participation is voluntary, refusal to participate will involve no penalty or loss of benefits to which the subject is otherwise entitled, and the subject may discontinue participation at any time without penalty or loss of benefits to which the subject is otherwise entitled."

322. The Belmont Report, having the force of law,[117] declares, "An agreement to participate in research constitutes a valid consent only if voluntarily given. This element of informed consent requires conditions free of coercion and undue influence" and "Respect for persons requires that subjects, to the degree that they are capable, be given the opportunity to choose what shall or shall not happen to them. This opportunity is provided when adequate standards for informed consent are satisfied."

323. Defendants breached their duties to establish "adequate standards" of informed consent when applying "sanctions," "coercion," "undue influence," and "unjustifiable pressures" on Plaintiffs to participate in COVID-19 investigational new drugs and devices (e.g., masks, testing articles). At all times pertinent, Defendants did not obtain Plaintiffs' legally effective informed consent.

324. Article VII of the ratified International Covenant on Civil and Political Rights (ICCPR) Treaty affirms that "…no one shall be subjected without his free consent to medical or scientific experimentation."

325. The Defendants' actions described above, individually and/or collectively, acting under color of state law, and in deprivation of the Constitutional rights and rights secured by the above federal statutes, regulations, and treaty, unlawfully subjected Plaintiffs to the use of investigational medical products under threat of penalty outside of their legally effective informed

[117] 45 CFR § 46.101(c), 45 CFR 46.101(i), 45 CFR § 46.122

consent as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT TWO

42 U.S.C. § 1983 – Deprivation of Equal Protection Rights

326. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

327. The CDC COVID-19 Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR Part 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

328. The Fourteenth Amendment to the U.S. Constitution guarantees equal protection of the laws.

329. At all times pertinent, Defendants intentionally only penalized individuals who exercised their federally secured right to refuse administration of a product under the PREP Act or an EUA drug (e.g., Pfizer-BioNTech COVID-19 Vaccine), biologic, or device (e.g., masks, COVID-19 testing articles) thereby applying the laws unequally to and depriving Plaintiffs, of their Constitutional Equal Protection Rights.

330. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their equal protection rights as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT THREE

42 U.S.C. § 1983 – Deprivation of Constitutional Due Process Rights

331. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

332. The CDC COVID-19 Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

333. The Due Process Clause of the Fourteenth Amendment to the U.S. Constitution guarantees the right to due process of law before infringing a citizen's interest in life, liberty, or property.

334. At all times pertinent, Defendants, having knowledge of Plaintiffs' Constitutional and federally secured right to refuse administration of EUA drugs and medical products, intentionally ignored those rights in an attempt to increase the number of participants in the CDC COVID-19 Vaccination Program for purposes of greed, resulting in the deprivation of Plaintiffs' substantive and procedural Due Process rights.

335. "The fundamental requisite of due process of law is the opportunity to be heard." *Louisville & Nashville R. Co. v. Schmidt*, 177 U. S. 230, 177 U. S. 236. Defendants did not provide Plaintiffs with a date, time, place, or procedure to defend their right to refuse injection of an unlicensed drug before depriving them of their liberty and property.

336. Defendants' requirement that Plaintiffs inject unlicensed drugs into their bodies as a condition to sell their labor "is not a legitimate exercise of the police power of the State, but an

unreasonable, unnecessary and arbitrary interference with the right and liberty of the individual to contract in relation to labor, and, as such, it is in conflict with, and void under, the Federal Constitution." *Lochner v. New York*, 198 U.S. 45 (1905)

337. Plaintiffs have the Constitutional right "to present [their] case and have its merits fairly judged." *Logan v. Zimmerman Brush Co*., 455 U.S. 422 (1982). At all times pertinent, Defendants refused to acknowledge Plaintiffs' Constitutional and Statutory rights, thereby nullifying impartiality.

338. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, have deprived the Plaintiffs of their substantive and procedural due process rights as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT FOUR

42 U.S.C. § 1983 - Deprivation of Rights Under the Spending Clause

339. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

340. The laws cited in the CDC COVID-19 Vaccination Program Provider Agreement, 45 CFR §46.122, 10 U.S.C. §980, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

341. In *Gonzaga Univ. v. Doe*, 536 U.S. 273 (2002), "the Court has found that spending legislation gave rise to rights enforceable under §1983 only in *Wright* v. *Roanoke Redevelopment and Housing Authority,* 479 U. S. 418, 426, 432, and *Wilder* v. *Virginia Hospital Assn.,* 496 U. S. 498, 522523, where statutory provisions explicitly conferred specific monetary entitlements upon the plaintiffs, and there was no sufficient administrative means of enforcing the requirements

against defendants that failed to comply." See also, *Health and Hospital Corporation of Marion County v. Talevski*, *supra*, 599 U.S. ____ (2023)

342. The federal government appropriated funds to the Department of Defense to enter into contracts with the manufacturers of the EUA investigational drugs to purchase 100% of the products and to distribute them to the Organizations that signed the CDC COVID-19 Vaccination Program Provider Agreement.

343. The federal government funds any charges associated with the administration of the COVID-19 EUA shots via Medicare.[118]

344. In the case at bar, the "specific monetary entitlement" to Plaintiffs, and any potential recipient, is that the EUA investigational drugs and their administrative costs are free of charge to the recipients.

345. 45 CFR §46.122 provides: "Federal funds administered by a Federal department or agency may not be expended for research involving human subjects unless the requirements of this policy have been satisfied."

346. 10 U.S.C. §980 states: "Funds appropriated to the Department of Defense may not be used for research involving a human being as an experimental subject unless – the subject's informed consent is obtained in advance…"

347. Only Organizations who agreed to participate in the CDC Vaccination Program can bill the government for administering the shots.

348. The EUA statute and the PREP Act lack any enforcement scheme for a breach of a potential recipient's right to refuse administration of an EUA investigational drug without penalty that would preclude §1983 enforcement.

[118] https://www.medicare.gov/medicare-coronavirus

349. Agreement Requirement Number 3 on the CDC Provider Agreement states, "Organization must not sell or seek reimbursement for COVID-19 Vaccine and any adjuvant, syringes, needles, or other constituent products and ancillary supplies that the federal government provides without cost to Organization."

350. Agreement Requirement Number 4 states, "Organization must administer COVID-19 Vaccine regardless of the vaccine recipient's ability to pay COVID-19 Vaccine administration fees."

351. These two provisions establish a specific monetary entitlement to the individual.

352. Agreement Requirement Number 5 states, "Before administering COVID-19 Vaccine, Organization must provide an approved Emergency Use Authorization (EUA) Fact Sheet or vaccine information statement (VIS), as required, to each vaccine recipient, the adult caregiver accompanying the recipient, or other legal representative."

353. Agreement Requirement Number 5 complies with funding restrictions established by Congress in, 45 CFR § 46.122 and 10 U.S.C. § 980.

354. The compliance is found in the EUA Fact Sheet, notating the individual's right to refuse the administration of the product. This express right is the fundamental requirement in obtaining the legally effective informed consent of the individual.

355. Whether for civilians under 45 CFR § 46.122 or personnel under 10 U.S.C. § 980, Congress created a specific monetary entitlement for individuals considering whether or not to participate in a federally funded research activity. That entitlement means they have the explicit right to be informed of the risks, benefits, and alternatives to the research product and then consider whether to participate without incurring a fee or being under outside pressure to participate.

356. This monetary entitlement is most apparent in the CDC COVID-19 Vaccination Program Provider Agreement. An individual can seek out a participating COVID-19 Program healthcare professional, obtain medical counseling, ask questions, and read literature. If they choose not to participate, they will not incur a fee from the professional for the administrative time spent considering whether or not to participate since the healthcare professional must inform them of their legal right to refuse under 21 U.S.C. §360bbb-3.

357. The healthcare professional agreed to comply with the legally effective consent requirements via Agreement Number 12 on the CDC COVID-19 Vaccination Program Provider Agreement mandating that (1) "Organization must comply with all applicable requirements as set forth by the U.S. Food and Drug Administration, including but not limited to requirements in any EUA that covers COVID-19 Vaccine," and (2) "Organization must administer COVID-19 Vaccine in compliance with all applicable state and territorial vaccination laws."

358. The "all applicable requirements as set forth by the U.S. Food and Drug Administration, including…any EUA" extends to 21 USC 360bbb-3 (Section 564), 45 CFR 46, the FWA, the IRB, the ICCPR Treaty, and the Scope of Authorization letter.

359. Defendants were under explicit legal obligations to comply with 45 CFR § 46.122, 10 U.S.C. § 980 via their FWA agreement and their CDC COVID-19 Vaccination Program participation.

360. Therefore, the laws cited in CDC COVID-19 Vaccination Program Provider Agreement, 21 U.S.C. §360bbb-3, 45 CFR § 46.122, and 10 U.S.C. §980 clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983 when federal funds are expended under those provisions of law.

361. The Defendants' actions described above, individually and/or collectively, and in deprivation of the rights and privileges secured by the Constitution and the above statutes and regulations, refused to obtain the legally effective informed consent of the Plaintiffs in violation of spending legislation as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT FIVE

Unconstitutional Conditions Doctrine - 42 U.S.C. § 1983

362. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

363. The CDC COVID-19 Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR §46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

364. "…the state, having power to deny a privilege altogether, may grant it upon such conditions as it sees fit to impose. But the power of the state in that respect is not unlimited; and one of the limitations is that it may not impose conditions which require the relinquishment of constitutional rights. If the state may compel the surrender of one constitutional right as a condition of its favor, it may, in like manner, compel a surrender of all. It is inconceivable that guaranties embedded in the Constitution of the United States may thus be manipulated out of existence (emphasis added)". *Frost Trucking Co. v. R.R. Com*, 271 U.S. 583, 593-94 (1926)

365. Governor Inslee and Shriners PolicyMakers established conditions requiring Plaintiffs to surrender their Constitutional rights under the Fourteenth Amendment to enjoy privileges of the State, such as the ability to sell their labors in the marketplace freely.

368. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, manipulated the Constitutional rights of Plaintiffs out of existence as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT SIX

PREP Act - 42 U.S.C. § 1983

369. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

370. The PREP Act, the CDC COVID-19 Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR Part 46, the Belmont Report, 21 U.S.C. §360bbb-3, Article VII of the ICCPR Treaty, Federal Wide Assurance, the EUA Scope of Authorization letter, and the Fourteenth Amendment clearly and unambiguously create rights enforceable pursuant to 42 U.S.C. § 1983.

371. The PREP Act provides certain immunities to "covered countermeasures" when the HHS Secretary determines there is a public health emergency and makes a declaration of that emergency through the publication in the Federal Register specifying the conditions by which the covered countermeasure and covered persons can participate and the use of such covered countermeasure.[119]

[119] 42 USC 247d-6d(b)(1)

372. Congress preempted the State of Washington and medical facilities it licenses from establishing laws and continuing in effect with existing ones (at-will employment doctrine) that would otherwise interfere with Plaintiffs' authority with respect to "conduct undertaken" concerning "any matter included in a requirement applicable" to a "covered countermeasure" under the PREP Act or 21 U.S.C. §360bbb-3 including the required condition that Plaintiffs be informed of their legal right to either accept or refuse said countermeasure.[120],[121]

373. Congress was explicit that the HHS Secretary must establish conditions ensuring that "potential participants are educated with respect to…the voluntary nature of the program…"[122]

374. The "program" consists of those agreeing to manufacture, distribute, administer ("covered person"), and receive[123] ("covered individual") the product.

375. Congress expressly restricted the HHS Secretary from having any authority to require any person to participate in any activity involving a "drug," "biologic," or "device" under 21 U.S.C. §360bbb-3[124] or any "covered countermeasure" under the PREP Act. By extension, any person authorized to participate in the program is also restricted from mandating participation.

376. The State of Washington and Shriners PolicyMakers established laws and policies that conflicted with the PREP Act and 21 U.S.C. §360bbb-3 when they required Plaintiffs to participate in the use of a covered countermeasure under threat of penalty. Moreover, Defendants engaged in policy-making and conduct that conflicted with the PREP Act and the Fifth and Fourteenth Amendments of the United States Constitution.

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[120] 21 U.S.C. 301 is the Federal Food, Drug and Cosmetic Act, which ranges from §301 to §399, and thus includes 21 U.S.C. §360bbb-3(e)(1)(A)(ii)(III).
[121] 42 USC 247d-6d(b)(8)
[122] 42 USC 247d-6e(c)
[123] 42 U.S.C. §247d-6e(e)(2)
[124] 21 U.S.C. §360bbb-3(l) " –Nothing in this section provides the Secretary any authority to require any person to carry out any activity that becomes lawful pursuant to an authorization under this section…"

377. Mandatory participation in PREP Act covered countermeasures is a severe violation of the Constitution's Due Process guarantees.

378. No person can be required to enter into a legally binding agreement requiring the forfeiture of legal rights under threat of penalty.

379. The terms and conditions associated with the PREP Act and 21 U.S.C. §360bbb-3 represent a legally binding agreement as established by the U.S. Congress. Those terms require Plaintiffs to forfeit their right to seek judicial relief from injuries sustained from the use of the countermeasure and injuries sustained from the countermeasure's administration. The agreement also requires Plaintiffs to divulge their private health information and private identity and assume greater risks to their health, safety, and legal rights.

380. Defendants' pronouncement that Plaintiffs must participate in covered countermeasures prospectively denies Plaintiffs their due process rights should they incur injury because the PREP Act denies them access to judicial relief for those injuries.

381. Governor Inslee's and Shriners PolicyMakers' policies violated Plaintiffs' rights to accept or refuse participation in PREP Act covered countermeasures without pressure or influence being placed upon them.

382. Governor Inslee's and Shriners PolicyMakers' issued policies requiring participation in investigational drugs, testing articles, masks, and other devices under threat of penalty, violating Plaintiffs' right to voluntary participation and due process rights.

383. Defendants changing the voluntary nature of the program into an involuntary program endangers the immunities of existing covered countermeasures established by the HHS Secretary. Defendants' interference is a direct assault on the Constitutional rights of Plaintiffs, which opens the doors to legal remedies not envisioned by Congress but required of the

Constitution for resulting injuries sustained by individuals when under threat of penalty to participate.

384. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty, deprived the Constitutional and federal legal rights of Plaintiffs as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT SEVEN

Breach of Contract, Third Party Beneficiary

385. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

386. The CDC COVID Vaccination Program Provider Agreement, and the implementing statutes and regulations found at 45 CFR 46, 21 U.S.C. §360bbb-3, Title 21 of the US Code, the EUA Scope of Authorization letter clearly and unambiguously create third-party beneficiary rights.

387. The primary third-party beneficiary right intended for Plaintiffs is the freedom to consider participation in a federally funded EUA (drug, biologic, or device), PREP Act, or other emergency medical countermeasure products or activities that are free from "sanctions," "coercion," "undue influence," "unjustifiable pressures to participate. The other third-party benefit intended for Plaintiffs is that they must not fear the loss of benefits to which they are otherwise entitled when considering participation. Defendants issued a policy that was an "overt threat of harm"[125] to the financial and emotional well-being of Plaintiffs for the express purpose of coercing

[125]" Coercion occurs when an overt threat of harm is intentionally presented by one person to another in order to obtain compliance." — The Belmont Report

them to participate in the CDC COVID-19 Vaccination Program outside of their free will and voluntary consent.

388. The Defendants' actions described above, individually and/or collectively, and in breach of the CDC COVID-19 Vaccination Program Provider Agreement, deprived the Plaintiffs of the benefits intended to be conferred upon them through the terms and conditions of the CDC COVID Vaccination Program Provider Agreement as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT EIGHT

Washington State Common Law Employment Torts

389. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

390. The Supremacy Clause, PREP Act, and 21 U.S.C. §360bbb-3 preempt State laws conflicting with the United States Government's emergency medical countermeasure objectives, including Washington State's at-will employment laws.

391. Defendants lacked authority to condition employment upon Plaintiffs participating in a 21 U.S.C. §360bbb-3 medical countermeasure or any product or activity under PREP Act authority.

392. Defendants intentionally and unlawfully misrepresented their authority to Plaintiffs to cause them to surrender their constitutional and statutory rights.

393. Defendants engaged in acts of coercion, undue influence, and retaliation, creating a hostile work environment.

394. Defendants placed Plaintiffs under moral duress[126], knowing they exclusively relied on Defendants for access to living wages.

395. Defendants segregated Plaintiffs under discriminatory acts upon Plaintiffs exercising their absolute right to refuse investigational new drugs.

396. Defendants unlawfully altered Plaintiffs' employment schedules under coercive acts to punish them for exercising their absolute right to refuse investigational new drugs.

397. Defendants attempted to coerce Plaintiffs to waive their federally secured right to refuse EUA investigational products and engage in a legally binding agreement under the terms and conditions (21 U.S.C. §360bbb-3 and PREP Act) established by the United States Congress outside their free will and voluntary consent.

398. Defendants' actions demonstrate moral turpitude against Plaintiffs' rights, safety, and health.

399. Defendants willfully and intentionally placed Plaintiffs under historic public and private pressure to enter into a legally binding agreement outside of their free will and voluntary consent.

400. Defendants unlawfully terminated Plaintiffs' employment when Plaintiffs exercised a legal right or privilege.[127]

401. The plaintiffs did not knowingly submit to the deprivation of labor, wages, or employment.

[126] Moral duress consists of imposition, oppression, undue influence, or the taking of undue advantage of the business or financial stress or extreme necessity or weakness of another. *Lafayette Dramatic Productions v. Ferentz*, 306 Mich. 193, 9 N.W.2d 57, 66; See also Black's Law Dictionary, Sixth Edition, p. 1008.

[127] *Becker v. Community Health Systems*, 184 Wn.2d 252, 359 P.3d 746 (2015); *Rose v. Anderson Hay & Grain Co*., 184 Wn.2d 268, 358 P.3d 1159 (2015), and *Rickman v. Premera Blue Cross*, 184 Wn.2d 300, 358 P.3d 1153 (2015)

402. The Defendants' actions, individually and/or collectively, and in derogation of Washington State's common laws, deprived the intended benefits conferred upon the Plaintiffs when enjoying employment in the State of Washington as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

COUNT NINE

Outrage

403. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

404. When the United States Congress refused to allow, Defendants, to apply consequences to Plaintiffs refusing to participate in the use of COVID-19 investigational drugs, Defendants engaged in a scorched earth policy and inflicted with malicious intent severe emotional distress to the fullest extent that one in their positions of authority and power could inflict to the detriment of Plaintiffs' emotional well-being.

405. Outrage[128] is proven by (1) demonstrating that Defendants engaged in extreme and outrageous conduct, (2) Defendants' actions caused severe emotional distress, (3) Defendants intentionally or recklessly caused the emotional distress, (4) that Plaintiffs are the direct recipients of the conduct.[129]

406. The facts and the Defendants' conduct committed with gross negligence, reckless, or intent, as described above in the complaint, give rise to a claim of Outrage under the common

[128] "Outrage" and "intentional infliction of emotional distress" are synonyms for the same tort. See *Snyder v. Med. Serv. Corp*., 145 Wn.2d 233, 250, 35 P.3d 1158 (2001); *Kloepfel v. Bokor*, 149 Wn. 2d 192, 194 n.1 (Wash. 2003)
[129] *Kloepfel v. Bokor*, 149 Wn.2d 192, 195, 66 P.3d 630 (2003); *Robel v. Roundup Corp*., 148 Wn.2d 35, 64, 59 P.3d 611 (2002); *Reid v. Pierce County*, 136 Wn.2d 195, 202, 961 P.2d 333 (1998) (citing *Dicomes v. State*, 113 Wn.2d 612, 630, 782 P.2d 1002 (1989)); *Kirby v. City of Tacoma*, 124 Wn.App. 454, 473, 98 P.3d 827 (2004).

law of the State of Washington against the Defendants for the damages described in Paragraphs 410 through 416, *infra*.

COUNT TEN

Implied Private Right of Action 21 U.S.C. §360bbb-3

407. Plaintiffs incorporate by reference the allegations contained in paragraphs 1 through 314, as if fully set forth herein.

408. Should the court not agree that Shriners PolicyMakers was engaged in State Action, Plaintiffs claim that 21 U.S.C. §360bbb-3 contains an implied private right of action pursuant to *Cannon v. University of Chicago*, 441 U.S. 677 (1979), *Wilder v. Virginia Hosp. Ass'n*, 496 U.S. 498 (1990), and *Cort v. Ash*, 422 U.S. 66 (1975).

409. The Defendants' actions described above, individually and/or collectively, and in derogation of the Constitution and the above statutes, regulations, and treaty have deprived the Plaintiffs of their explicit right to refuse the administration of an emergency use authorized drug and/or medical product without penalty as described in the above facts, thereby causing them damages described in Paragraphs 410 through 416, *infra*.

VIII. Damages Recoverable and Demanded

410. The following paragraphs are hereby incorporated by reference into Counts One through Ten, as if set forth here *in extenso*.

411. As a direct and proximate result of the Defendants' unreasonable and unlawful actions, Plaintiffs have suffered past damages and will suffer future damages, both compensatory and general, including, but not limited to, front and back pay; loss of benefits; loss of accumulated sick pay; loss of retirement accounts; lost earnings on retirement funds; vacation time, compensatory time, and paid time off; negative tax consequences (in the event of a lump sum

award), including related accountant fees; attorney's fees; emotional distress; mental, psychological and physical harm; loss of income; loss of enjoyment of life; for which defendants are liable in compensatory, punitive, exemplary, legal, equitable, and all other damages that this Court deems necessary and proper.

412. When the Defendants' behavior reaches a sufficient threshold, punitive damages are recoverable in § 1983 cases. *Smith v. Wade*, 461 U.S. 30 (1983). Because Defendants' actions were intentional and willful, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each and every Defendant in an amount sufficient to deter them, individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

413. Because Defendants' actions involved reckless or callous indifference to the Plaintiffs' federally protected rights, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each and every Defendant in an amount sufficient to deter them, individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

414. Because Defendants' actions were motivated by evil motive or intent, Plaintiffs are entitled to, and hereby demand, an award of punitive damages against each and every Defendant in an amount sufficient to deter them, individually and collectively, from repeating their unconstitutional actions. *Smith v. Wade*, 461 U.S. 30 (1983)

415. Plaintiffs seek recovery of attorney's fees under the Civil Rights Attorney's Fees Awards Act of 1976 and 42 U.S.C. § 1988, and under any other provision of law or basis.

416. Plaintiffs seek recovery of all court costs and out-of-pocket litigation expenses, including but not limited to expert fees, and legal interest on any amount of damages awarded.

IX. Jury Trial Demand

417. Plaintiffs are entitled to, and hereby demand, a trial by jury on all issues of fact.

WHEREFORE, Plaintiffs pray that Defendants be served with a copy of this Complaint and be duly cited to appear and answer same, and after due proceedings are had, there be judgment herein against the Defendants awarding Plaintiffs all damages claimed herein, plus legal interest, taxable costs, expert fees, and attorney's fees, and all other relief determined to be just and equitable by this Court.

Respectfully Submitted,

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